

CONSOLIDATED FINANCIAL STATEMENTS

Memorial Hermann Health System
Years Ended June 30, 2024 and 2023
With Report of Independent Auditors



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Memorial Hermann Health System

Consolidated Financial Statements

Years Ended June 30, 2024 and 2023

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Report of Independent Auditors

Management and the Audit Committee of the Board of Directors
Memorial Hermann Health System

Report on the Audit of the Financial Statements

Opinion

We have audited the consolidated financial statements of Memorial Hermann Health System (the Health System), which comprise the consolidated balance sheets as of June 30, 2024 and 2023, and the related consolidated statements of operations and changes in net assets and cash flows for the years then ended and the related notes (collectively referred to as the “financial statements”).

In our opinion, the accompanying financial statements present fairly, in all material respects, the financial position of the Health System at June 30, 2024 and 2023, and the results of its operations, changes in its net assets and its cash flows for the years then ended in accordance with accounting principles generally accepted in the United States of America.

Basis for Opinion

We conducted our audits in accordance with auditing standards generally accepted in the United States of America (GAAS) and the standards applicable to financial audits contained in *Government Auditing Standards* issued by the Comptroller General of the United States (*Government Auditing Standards*). Our responsibilities under those standards are further described in the Auditor’s Responsibilities for the Audit of the Financial Statements section of our report. We are required to be independent of the Health System, and to meet our other ethical responsibilities, in accordance with the relevant ethical requirements relating to our audits. We believe that the audit evidence we have obtained is sufficient and appropriate to provide a basis for our audit opinion.

Responsibilities of Management for the Financial Statements

Management is responsible for the preparation and fair presentation of the financial statements in accordance with accounting principles generally accepted in the United States of America, and for the design, implementation, and maintenance of internal control relevant to the preparation and fair presentation of financial statements that are free of material misstatement, whether due to fraud or error.



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In preparing the financial statements, management is required to evaluate whether there are conditions or events, considered in the aggregate, that raise substantial doubt about the Health System's ability to continue as a going concern for one year after the date that the financial statements are issued.

Auditor's Responsibilities for the Audit of the Financial Statements

Our objectives are to obtain reasonable assurance about whether the financial statements as a whole are free of material misstatement, whether due to fraud or error, and to issue an auditor's report that includes our opinion. Reasonable assurance is a high level of assurance but is not absolute assurance and therefore is not a guarantee that an audit conducted in accordance with GAAS and *Government Auditing Standards* will always detect a material misstatement when it exists. The risk of not detecting a material misstatement resulting from fraud is higher than for one resulting from error, as fraud may involve collusion, forgery, intentional omissions, misrepresentations, or the override of internal control. Misstatements are considered material if there is a substantial likelihood that, individually or in the aggregate, they would influence the judgment made by a reasonable user based on the financial statements.

In performing an audit in accordance with GAAS and *Government Auditing Standards*, we:

- Exercise professional judgment and maintain professional skepticism throughout the audit.
- Identify and assess the risks of material misstatement of the financial statements, whether due to fraud or error, and design and perform audit procedures responsive to those risks. Such procedures include examining, on a test basis, evidence regarding the amounts and disclosures in the financial statements.
- Obtain an understanding of internal control relevant to the audit in order to design audit procedures that are appropriate in the circumstances, but not for the purpose of expressing an opinion on the effectiveness of the Health System's internal control. Accordingly, no such opinion is expressed.
- Evaluate the appropriateness of accounting policies used and the reasonableness of significant accounting estimates made by management, as well as evaluate the overall presentation of the financial statements.
- Conclude whether, in our judgment, there are conditions or events, considered in the aggregate, that raise substantial doubt about the Health System's ability to continue as a going concern for a reasonable period of time.



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We are required to communicate with those charged with governance regarding, among other matters, the planned scope and timing of the audit, significant audit findings, and certain internal control-related matters that we identified during the audit.

Supplementary Information

Our audits were conducted for the purpose of forming an opinion on the financial statements as a whole. The accompanying Schedule of Expenditures of Federal Awards as required by Title 2 U.S. Code of Federal Regulations Part 200, *Uniform Administrative Requirements, Cost Principles, and Audit Requirements for Federal Awards* is presented for purposes of additional analysis and is not a required part of the financial statements. Such information is the responsibility of management and was derived from and relates directly to the underlying accounting and other records used to prepare the financial statements. The information has been subjected to the auditing procedures applied in the audit of the financial statements and certain additional procedures, including comparing and reconciling such information directly to the underlying accounting and other records used to prepare the financial statements or to the financial statements themselves, and other additional procedures in accordance with GAAS. In our opinion, the information is fairly stated, in all material respects, in relation to the financial statements as a whole.

Other Reporting Required by *Government Auditing Standards*

In accordance with *Government Auditing Standards*, we have also issued our report dated September 25, 2024 on our consideration of the Health System's internal control over financial reporting and on our tests of its compliance with certain provisions of laws, regulations, contracts, and grant agreements and other matters. The purpose of that report is solely to describe the scope of our testing of internal control over financial reporting and compliance and the results of that testing, and not to provide an opinion on the effectiveness of the Health System's internal control over financial reporting or on compliance. That report is an integral part of an audit performed in accordance with *Government Auditing Standards* in considering the Health System's internal control over financial reporting and compliance.

Ernst & Young LLP

September 25, 2024, except for our report
on the Schedule of Expenditures of Federal
awards for which the date is March 25, 2025



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Report of Independent Auditors on Internal Control Over Financial Reporting and on Compliance and Other Matters Based on an Audit of Financial Statements Performed in Accordance with *Government Auditing Standards*

Management and the Audit Committee of the Board of Directors
Memorial Hermann Health System

We have audited, in accordance with auditing standards generally accepted in the United States of America and the standards applicable to financial audits contained in *Government Auditing Standards* issued by the Comptroller General of the United States (*Government Auditing Standards*), the consolidated financial statements of Memorial Hermann Health System, which comprise the consolidated balance sheets as of June 30, 2024 and 2023, and the related consolidated statements of operations and changes in net assets and cash flows for the years then ended, and the related notes to the financial statements, and have issued our report thereon dated September 25, 2024.

Report on Internal Control Over Financial Reporting

In planning and performing our audit of the financial statements, we considered Memorial Hermann Health System's internal control over financial reporting (internal control) as a basis for designing audit procedures that are appropriate in the circumstances for the purpose of expressing our opinion on the financial statements, but not for the purpose of expressing an opinion on the effectiveness of Memorial Hermann Health System's internal control. Accordingly, we do not express an opinion on the effectiveness of Memorial Hermann Health System's internal control.

A deficiency in internal control exists when the design or operation of a control does not allow management or employees, in the normal course of performing their assigned functions, to prevent, or detect and correct misstatements on a timely basis. A *material weakness* is a deficiency, or a combination of deficiencies, in internal control, such that there is a reasonable possibility that a material misstatement of the entity's financial statements will not be prevented, or detected and corrected on a timely basis. A *significant deficiency* is a deficiency, or a combination of deficiencies, in internal control that is less severe than a material weakness, yet important enough to merit attention by those charged with governance.



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Our consideration of internal control was for the limited purpose described in the first paragraph of this section and was not designed to identify all deficiencies in internal control that might be material weaknesses or significant deficiencies. Given these limitations, during our audit we did not identify any deficiencies in internal control that we consider to be material weaknesses. However, material weaknesses or significant deficiencies may exist that were not identified.

Report on Compliance and Other Matters

As part of obtaining reasonable assurance about whether Memorial Hermann Health System's financial statements are free of material misstatement, we performed tests of its compliance with certain provisions of laws, regulations, contracts and grant agreements, noncompliance with which could have a direct and material effect on the financial statements. However, providing an opinion on compliance with those provisions was not an objective of our audit, and accordingly, we do not express such an opinion. The results of our tests disclosed no instances of noncompliance or other matters that are required to be reported under *Government Auditing Standards*.

Purpose of This Report

The purpose of this report is solely to describe the scope of our testing of internal control and compliance and the results of that testing, and not to provide an opinion on the effectiveness of the entity's internal control or on compliance. This report is an integral part of an audit performed in accordance with *Government Auditing Standards* in considering the entity's internal control and compliance. Accordingly, this communication is not suitable for any other purpose.

Ernst & Young LLP

September 25, 2024

Memorial Hermann Health System

Consolidated Balance Sheets (In Thousands)

	June 30	
	2024	2023
Assets		
Current assets:		
Cash and cash equivalents (includes \$147,000 and \$165,542 of restricted cash in 2024 and 2023, respectively)	\$ 798,690	\$ 751,511
Patient accounts receivable	963,434	795,884
Other current assets	678,666	526,340
Total current assets	2,440,790	2,073,735
Investments	4,950,885	4,487,464
Assets limited as to use, less current portion	471,085	267,314
Property, plant, and equipment, net	3,449,857	3,179,389
Operating lease right-of-use assets, net	476,489	493,262
Other assets	330,949	273,878
Total assets	<u>\$ 12,120,055</u>	<u>\$ 10,775,042</u>
Liabilities and net assets		
Current liabilities:		
Accounts payable	\$ 373,686	\$ 309,787
Accrued payroll and related expenses	423,454	381,262
Other accrued expenses	367,296	332,408
Current portion of long-term debt, including finance lease obligations	78,434	65,261
Long-term debt subject to liquidity agreements	331,802	455,609
Current portion of operating lease right-of-use obligations	76,575	77,921
Total current liabilities	1,651,247	1,622,248
Long-term debt, including finance lease obligations	2,408,278	1,868,574
Operating lease right-of-use obligations	422,280	437,036
Other long-term obligations	262,056	304,971
Total liabilities	4,743,861	4,232,829
Net assets without donor restrictions, including non-controlling interests of \$161,833 and \$163,594 at June 30, 2024 and 2023, respectively	7,216,123	6,405,331
Net assets with donor restrictions	160,071	136,882
Total net assets	7,376,194	6,542,213
Total liabilities and net assets	<u>\$ 12,120,055</u>	<u>\$ 10,775,042</u>

See accompanying notes.

Memorial Hermann Health System

Consolidated Statements of Operations and Changes in Net Assets (In Thousands)

	Year Ended June 30	
	2024	2023
Without donor restrictions		
Revenues, gains, and other support:		
Net patient service revenue	\$ 8,203,861	\$ 7,463,674
Other revenue	440,334	406,608
Total revenues, gains, and other support	8,644,195	7,870,282
Expenses:		
Salaries, benefits, and related personnel costs	3,682,189	3,407,705
Services and other	2,644,053	2,232,408
Supplies and medicines	1,512,237	1,427,289
Depreciation and amortization	385,974	368,805
Interest	111,461	110,973
Total expenses	8,335,914	7,547,180
Operating income	308,281	323,102
Non-operating activities:		
Investment gains and other, net	468,595	384,740
Interest rate swap agreements	16,668	8,292
Gain on extinguishment of debt	10,839	—
Recognition of deferred gain on sale of assets	62,978	—
Revenues in excess of expenses	867,361	716,134
Revenues in excess of expenses attributable to non-controlling interests	(96,201)	(85,281)
Revenues in excess of expenses attributable to the Health System	771,160	630,853
Other changes in net assets:		
Change in pension asset	11,559	(17,847)
Contributions and grants received and other changes in net assets, net	29,834	7,269
Change in non-controlling interests	(1,761)	13,686
Change in net assets without donor restrictions	810,792	633,961
With donor restrictions		
Contributions	50,976	26,864
Investment income, net	4,019	3,265
Net assets released from restrictions for operations, capital purchases, and other	(31,806)	(20,169)
Change in net assets with donor restrictions	23,189	9,960
Change in net assets	833,981	643,921
Net assets at beginning of year	6,542,213	5,898,292
Net assets at end of year	\$ 7,376,194	\$ 6,542,213

See accompanying notes.

Memorial Hermann Health System

Consolidated Statements of Cash Flows (In Thousands)

	Year Ended June 30	
	2024	2023
Operating activities		
Cash received for patient services	\$ 8,001,246	\$ 7,466,397
Cash paid to or on behalf of employees	(3,617,558)	(3,401,331)
Cash paid for supplies and services	(4,069,044)	(3,725,126)
Other receipts from operations	381,511	370,540
Investment gains realized	279,049	120,467
Interest paid	(87,518)	(94,901)
Net cash provided by operating activities	<u>887,686</u>	<u>736,046</u>
Investing activities		
Capital expenditures, net	(516,147)	(415,491)
Change in assets limited as to use	(5,324)	17,452
Purchases of investments, net	(288,518)	(182,924)
Net cash used in investing activities	<u>(809,989)</u>	<u>(580,963)</u>
Financing activities		
Proceeds of issuance of long-term debt and note payable	952,877	17,355
Payments and defeasances on long-term debt and note payable	(739,555)	(81,629)
Restricted contributions	19,778	10,039
Non-controlling interests	(97,962)	(71,595)
Net cash provided by (used in) financing activities	<u>135,138</u>	<u>(125,830)</u>
Net increase in cash, cash equivalents, and restricted cash	212,835	29,253
Cash, cash equivalents, and restricted cash at beginning of year	797,374	768,121
Cash, cash equivalents, and restricted cash at end of year	<u>\$ 1,010,209</u>	<u>\$ 797,374</u>
Noncash investing and financing activities		
Recognition of lease liabilities arising from obtaining right-of-use assets under operating lease	\$ 46,683	\$ 34,826
Recognition of lease liabilities arising from obtaining right-of-use assets under finance lease	<u>\$ 109,993</u>	<u>\$ 46,425</u>

See accompanying notes.

Memorial Hermann Health System

Notes to Consolidated Financial Statements

June 30, 2024

1. Mission and Organization

Memorial Hermann Health System (the Health System), a Texas non-profit corporation, operates, directly and through affiliates, a multi-hospital, comprehensive health care system that has the largest market share in the Houston, Texas, greater metropolitan area. The Health System's Board of Directors exercises governance control for the Health System and retains significant reserved powers regarding its affiliates. The Health System is exempt from federal income tax under Section 501(c)(3) of the Internal Revenue Code (IRC).

The Health System currently operates 17 hospitals (including 11 general acute care hospitals, one children's hospital, one orthopedic and spine hospital, two rehabilitation hospitals, and two surgical hospitals), a comprehensive network of ambulatory care facilities, physician organizations and an independent practice association, a health plan that underwrites group health coverage, a captive casualty and liability insurance company, and an accountable care organization. Additionally, the Health System is supported by the Memorial Hermann Foundation. The accompanying consolidated financial statements include the accounts of the Health System and its controlled affiliates. All significant intercompany accounts and transactions have been eliminated.

2. Significant Accounting Policies

Use of Estimates

The preparation of financial statements in conformity with accounting principles generally accepted in the United States of America (U.S. GAAP) requires management to make estimates and assumptions that affect the reported amounts of assets, liabilities, revenues, expenses, and disclosure of contingent assets and liabilities at the date of the financial statements. Because of the subjectivity inherent in this process, actual results may differ from those estimates.

Subsequent Events

The Health System evaluates the impact of subsequent events, events that occur after the consolidated balance sheet date but before the consolidated financial statements are issued, for potential recognition in the consolidated financial statements as of the consolidated balance sheet date or disclosure in the notes to the consolidated financial statements. The Health System evaluated events occurring subsequent to June 30, 2024 through September 25, 2024, the date on which the accompanying consolidated financial statements were issued.

Memorial Hermann Health System

Notes to Consolidated Financial Statements (continued)

2. Significant Accounting Policies (continued)

Net Assets

To ensure compliance with restrictions placed on the resources available to the Health System, the Health System's accounts are maintained in accordance with the existence or absence of donor-imposed restrictions. Resources are classified for accounting and reporting into funds established according to their nature and purposes. In the accompanying consolidated financial statements, funds that have similar characteristics have been consolidated into two net asset categories:

- Net assets with donor restrictions are those net assets whose use has been restricted for a specific purpose or time period or those that stipulate the resources be maintained permanently but permit the Health System to use the income derived from the donated assets for donor-specified purposes.
- Net assets without donor restrictions are not restricted by donors or are those net assets whose donor-imposed restrictions have expired or been met. When a donor restriction expires, the net assets with donor restrictions are reclassified as net assets without donor restrictions and reported on the consolidated statements of operations and changes in net assets with donor restrictions as assets released from restrictions.

Unrestricted and restricted donations are recognized when received. Unrestricted and restricted pledges are reported as revenue in the period the pledge is made at the present value of estimated future cash flows. Amortization of the discount is included in contribution income. Pledges are recorded net of an allowance for uncollectible accounts. This allowance is determined based upon historical collection and write-off experience. Donor-restricted pledges and donations are recorded in the appropriate donor-restricted fund until restrictions are met, at which time they are contributed to the affiliate beneficiary. Gifts of property other than cash are recorded at fair market value at the dates the gifts are received.

The Health System holds donor-restricted endowment funds established primarily to fund specified activities for and within the Health System and the medical community as a whole. Net assets associated with endowment funds are classified and reported based on the existence or absence of donor-imposed restrictions.

Contributions are recorded as revenue at the present value of estimated future cash flows when an unconditional promise is received.

Memorial Hermann Health System

Notes to Consolidated Financial Statements (continued)

2. Significant Accounting Policies (continued)

Net Patient Service Revenue and Patient Accounts Receivable

Net Patient Service Revenue

The Health System's net patient service revenue is reported at the amount that reflects the consideration to which the Health System expects to be entitled in exchange for providing patient care. Revenues generally relate to contracts with patients in which performance obligations are to provide health care services to the patients. Revenues are recorded during the period that obligations to provide health care services are satisfied. Performance obligations for inpatient services are generally satisfied over periods that average approximately five days, and revenues are recognized based on charges incurred in relation to total expected charges. Performance obligations for outpatient services are generally satisfied over a period of less than one day. The contractual relationships with patients, in most cases, also involve a third-party payer (Medicare, Medicaid, managed care health plans, and commercial insurance companies), and the transaction prices for the services provided are dependent upon the terms provided by (Medicare and Medicaid) or negotiated with (managed care companies) the third-party payers. The payment arrangements with third-party payers for the services the Health System provides to the related patients typically specify payments at amounts less than the Health System's standard charges. Medicare generally pays for inpatient and outpatient services at prospectively determined rates based on clinical, diagnostic, and other factors. Services provided to patients having Medicaid coverage are generally paid at prospectively determined rates per discharge, per identified service, or per covered member. Agreements with managed care companies generally provide for payments based upon predetermined rates per diagnosis, per diem rates, or discounted fee-for-service rates.

The transaction price is determined based on gross charges for services provided, reduced by contractual adjustments provided to third-party payers, discounts provided to uninsured patients, and implicit price concessions provided primarily to uninsured patients. The Health System determines the estimates of adjustments and discounts based on contractual agreements, discount policies, and historical experience.

Management continually reviews the contractual estimation process to consider and incorporate updates to laws and regulations and the frequent changes in managed care contractual terms resulting from contract renegotiations and renewals. The estimate of implicit price concessions is based on the Health System's historical collection experience with these classes of patients using a portfolio approach as a practical expedient to account for patient contracts as collective groups rather than individually. The financial statement effects of using this practical expedient are not materially different from an individual contract approach.

Memorial Hermann Health System

Notes to Consolidated Financial Statements (continued)

2. Significant Accounting Policies (continued)

Amounts related to services provided to patients who were still receiving inpatient care in the Health System facilities at the end of the reporting period do not meet the conditions of unconditional right to payment and, therefore, are classified as contract assets. As of June 30, 2024 and 2023, the contract assets balance was \$144,000,000 and \$106,000,000, respectively, and is included in other current assets on the accompanying consolidated balance sheets.

The Health System's net patient service revenue by payer for the years ended June 30, 2024 and 2023, and approximate percentages of total, were as follows (dollar amounts in thousands):

	2024		2023	
	Amount	Ratio	Amount	Ratio
Managed care	\$ 3,968,569	48.4%	\$ 3,592,218	48.1%
Medicare	2,304,963	28.1	2,184,533	29.3
Medicaid	1,596,533	19.5	1,364,519	18.3
Self-pay and other	333,796	4.0	322,404	4.3
Total	\$ 8,203,861	100.0%	\$ 7,463,674	100.0%

The Health System's accounts receivable by payer as a percentage of net accounts receivable were as follows at June 30:

	2024	2023
Managed care	55%	56%
Medicare	22	21
Medicaid	12	12
Self-pay and other	11	11
Total	100%	100%

Medicare and Medicaid Programs

While laws and regulations governing the Medicare and Medicaid programs are complex and subject to interpretation, the Health System intends to be in compliance with all applicable laws and regulations and, to that end, has implemented a comprehensive organization-wide corporate compliance policy.

Memorial Hermann Health System

Notes to Consolidated Financial Statements (continued)

2. Significant Accounting Policies (continued)

Annual retroactive settlements with the Medicare and Medicaid programs are subject to review by appropriate governmental authorities or their agents. Settlements are accrued on an estimated basis in the period the related services are rendered and adjusted in future periods as final settlements are determined. Accruals for possible settlements are calculated based on historical experience.

The Health System's Medicare and Medicaid cost reports have been audited by the applicable fiscal intermediaries through December 31, 2019 for all hospitals and up to 2021 for certain entities. However, certain prior cost reports have been reopened by the Health System or by the fiscal intermediary. The reopenings are to expedite and accelerate the recovery of funds due to the Health System. Additionally, from time to time, the Health System appeals legal decisions in nature made by the fiscal intermediary in order to recover funds it believes are appropriately due to the Health System for services rendered to Medicare and/or Medicaid beneficiaries. The Health System's policy is to record any funds received from reopenings or appeals as income in the year in which the notice of cost report settlement is received.

As of June 30, 2024 and 2023, the Health System recorded net accruals and allowances for possible settlements, and pending reviews, as discussed above, of \$41,644,000 and \$33,653,000, respectively. It is reasonably possible that these estimates could differ from actual settlements and, thus, change in the near term by material amounts.

During 2024 and 2023, the Health System recognized \$21,521,000 and \$16,485,000, respectively, in net patient service revenue from differences between estimated and actual cost report settlements and appeals.

Medicaid Supplemental Payments

The Health System participates in several Medicaid supplemental payment programs, which are paid to hospitals providing a high volume of services to Medicaid and uninsured patients. These programs include:

- The Medicaid Disproportionate Share (DSH) hospital funding program was established by the state of Texas and administered by the Texas Health and Human Services Commission (THHSC). The program created additional federal matching funds to increase access to health care by Texas' indigent patients and defray the cost of treating indigent patients.

Memorial Hermann Health System

Notes to Consolidated Financial Statements (continued)

2. Significant Accounting Policies (continued)

- The 1115 Waiver program for uncompensated care (UC) was initially a five-year program beginning in fiscal year 2012; however, the program has been extended multiple times and has been approved until September 2030. Under the current waiver, to receive payments from the UC pool, a hospital must submit an application (referred to as the Waiver tool), estimating its uncompensated costs for services provided to Medicaid and uninsured patients.
- In March 2018, the Uniform Hospital Rate Increase Program (UHRIP) was implemented, under which hospital providers receive an additional payment for services provided to certain patients enrolled in Texas Medicaid managed care plans. The Comprehensive Hospital Increase Reimbursement Program (CHIRP) replaced the previous UHRIP program and provides increased Medicaid payments to hospitals for inpatient and outpatient services provided to persons enrolled in Texas Medicaid managed care plans. CMS has extended the program through August 31, 2024. THHSC has applied for an extension through August 31, 2025.
- The Medicaid Inpatient Direct Graduate Medical Education (GME) program established for state-owned or state-operated teaching hospitals was expanded during 2023 retroactive to state fiscal year 2019 to include private hospitals. In fiscal year 2024, THHSC approved and paid two retroactive payments for state fiscal year 2019 and 2022 and approved and paid for state fiscal year 2024 with final payment in July 2024. State fiscal years 2020, 2021, and 2023 are expected to be paid in state fiscal year 2025.
- The Medicaid program for the Private Hospital Augmented Reimbursement Program (HARP) was approved for private hospitals on August 15, 2023 retroactive to state fiscal year 2022. In fiscal year 2024, THHSC approved and paid one retroactive payment for state fiscal year 2022 and approved and paid for state fiscal year 2024. State fiscal year 2023 is expected to be paid in state fiscal year 2025.

The funding of these programs is dependent on intergovernmental transfers from state-owned and local governmental entities to draw down federal funds. Harris County Hospital District operates a local provider participation fund program (LPPF), which requires mandatory payments by all hospitals in the county based upon net patient revenue and the option of using the mandatory payment revenue as the non-federal share of supplemental Medicaid payments. Medicaid supplemental funds, which include DSH, CHIRP, HARP, GME and UC payments, of approximately \$1,143,449,000 and \$851,183,000, were recorded in net patient service revenue for the years ended June 30, 2024 and 2023, respectively. Services and other expenses include LPPF

Memorial Hermann Health System

Notes to Consolidated Financial Statements (continued)

2. Significant Accounting Policies (continued)

fees of \$473,385,000 and \$277,244,000 for the years ended June 30, 2024 and 2023, respectively. As of June 30, 2024 and 2023, the Health System recorded receivables of \$63,080,000 and \$13,829,000, respectively, and deferred revenues of \$114,337,000 and \$78,613,000, respectively, for Medicaid supplemental payment programs. These amounts are included in other current assets and other accrued expenses, respectively, on the accompanying consolidated balance sheets. Medicaid supplemental revenues have been recognized based on the most recent information available.

Premium Revenues, Claims Payable, and Medical Claims Expenses

Premium revenues are recognized in the month in which members are entitled to health care services. Premiums collected in advance are recorded as unearned premiums and are included in accrued liabilities on the accompanying consolidated balance sheets. Premium revenue of \$140,524,000 and \$115,441,000 was included in other revenue for the years ended June 30, 2024 and 2023, respectively.

The estimated cost of all health services rendered to members through June 30 but not yet paid as of that date, is included in other accrued expenses. This claims expense estimate is developed using actuarial assumptions based on historical experience with respect to the timing of payments in relation to the dates of service. Subsequent changes to prior period estimates are reflected in the current period. Losses on contracts are recognized in the period when health care costs are expected to exceed premium revenue.

Cash Equivalents

Liquid investments with an original maturity of three months or less are reported as cash equivalents, except for those held for long-term investing purpose or subject to restrictions, which are reported as investments or assets limited to use.

Memorial Hermann Health System

Notes to Consolidated Financial Statements (continued)

2. Significant Accounting Policies (continued)

The following table provides a reconciliation of cash, cash equivalents, and restricted cash reported within the consolidated balance sheets that sum to the total of the same such amounts shown on the consolidated statements of cash flows for the years ended June 30, 2024 and 2023:

	June 30	
	2024	2023
	<i>(In Thousands)</i>	
Cash and cash equivalents	\$ 651,690	\$ 585,969
Restricted cash	147,000	165,542
Investments	17,398	3,859
Assets limited as to use	194,121	42,004
Cash, cash equivalents, and restricted cash	<u>\$ 1,010,209</u>	<u>\$ 797,374</u>

Investments, Assets Limited to Use, and Investment Income

Investments are reflected as investments or assets limited as to use on the consolidated balance sheets and include fixed income, equity securities, master limited partnerships (MLPs), and alternative investments. Investments in equity securities and all debt securities are carried at fair value. Fixed income includes U.S. Government securities, mortgage-backed securities, asset-backed securities and other securitized credit, corporate obligations, and non-U.S. sovereign and corporate securities, in pooled funds and separate accounts. Equity securities include domestic and international equities in pooled funds and separate accounts. Pooled funds are professionally managed and include institutional mutual funds, fixed income funds, equity funds, and commingled accounts. Direct equity investments are investments in private companies and are valued at cost. MLPs include domestic MLPs in pooled funds and separate accounts and represent limited partnerships that are typically publicly traded. Other characteristics often associated with these MLPs include the requirement that the partnerships must generate most of their cash flows from particular businesses (including commodities).

Alternative investments include ownership interests in hedge funds and limited partnerships that may employ various investment strategies through the use of publicly traded securities, market neutral arbitrage, floating rate loans and debt securities, fixed income swaps, private real assets, and private equity. The Health System's alternative investments include certain investments which reported values had been estimated by fund managers in the absence of readily available market values or cannot otherwise be substantiated. Because of the inherent uncertainty of valuations,

Memorial Hermann Health System

Notes to Consolidated Financial Statements (continued)

2. Significant Accounting Policies (continued)

fund managers' estimates of fair value may differ from the values that would have been used had a ready market for the securities existed, and the differences could be material. Additionally, risks in certain of the Health System's alternative investments include limited transparency where funds are not required to disclose the holdings in their portfolios to the Health System and limitations on liquidity as funds may impose lockup periods or holdback provisions that limit the Health System's ability to redeem those investments. The Health System records its alternative investments at estimated fair value. As of June 30, 2024, the unfunded commitment related to alternative investments is approximately \$1,048,429,000. Investments in hedge funds have temporary redemptions frequency and notice period of 1 to 12 months with redemption restrictions of 1-3 years' gate provisions. Private equity and real assets, based on the nature of the fund, have a redemption frequency through the end of the life of the fund.

Investments are classified as non-current assets, regardless of their maturity date due to the Health System's primary intent not to utilize these assets to meet current obligations, capital, and other cash flow needs, and the investments have exposure to asset classes with longer-term investment horizons. Assets limited as to use are funds legally restricted by bond indentures, internally restricted in connection with self-insurance programs, externally restricted by donor specifications, restricted by resident agreements, or internally restricted for charity care or other purposes. Assets limited as to use are classified as non-current assets, except for assets limited as to use that are required to meet current liabilities, which are classified as current assets.

Substantially all of the Health System's investments are designated as trading investments. Investment income, including realized and unrealized gains and losses on investments, interest and dividend income, and equity in earnings of alternative investments, is recorded as a non-operating activity and included in revenues in excess of expenses on the accompanying consolidated statements of operations and changes in net assets, unless the income or loss is restricted by donor or law. Net purchases and sales of investments are reported as a component of net cash used in investing activities on the accompanying consolidated statements of cash flows, as the net proceeds were used primarily to fund the Health System's acquisition of capital assets.

The Health System maintains investments with various financial institutions and investment management firms, and its policy is designed to limit exposure to any one institution or investment, therefore reducing overall risk.

Memorial Hermann Health System

Notes to Consolidated Financial Statements (continued)

2. Significant Accounting Policies (continued)

Property, Plant, and Equipment

Property, plant, and equipment are carried at cost or fair value at the time of donation and include expenditures for new facilities and equipment and those expenditures that substantially increase the useful life of existing facilities and equipment. Ordinary maintenance and repairs are charged to expense when incurred. Depreciation is provided using the straight-line method over 20 to 40 years for buildings, 5 to 10 years for improvements (limited to the term of the related lease, if applicable), and 3 to 12 years for equipment. Assets accounted for as finance leases are amortized over the terms of the respective leases, and such amortization is included in depreciation and amortization expense.

When events, circumstances, or operating results indicate that the carrying values of certain long-lived assets might be impaired, the Health System prepares undiscounted projections of cash flows expected to result from the use of the assets and their eventual disposition. If the projections indicate that the recorded amounts are not expected to be recoverable, the related asset's carrying value is reduced to estimated fair value. Fair value may be estimated based upon internal evaluations that include quantitative analyses of revenues and cash flows, reviews of recent sales of similar assets, and independent appraisals. Property and equipment to be disposed of are reported at the lower of the carrying amounts or fair value less costs to sell or close. The estimates of fair value are usually based upon recent sales of similar assets and market responses based upon discussions with and offers received from potential buyers.

Interest Rate Swap Agreements

The Health System records its interest rate swap agreements at fair value on the consolidated balance sheets and the change in the fair values and net interest payments under swaps as a component of interest rate swap agreements on the consolidated statements of operations and changes in net assets.

Investments in Joint Ventures

The Health System has entered into multiple joint venture and partnership arrangements for the provision of medical services to patients. For those ventures where the Health System has a controlling interest through majority ownership, management control, or both, the ventures' assets, liabilities, and operating results have been included in the consolidated financial statements of the Health System. As of June 30, 2024 and 2023, the Health System has recognized net assets

Memorial Hermann Health System

Notes to Consolidated Financial Statements (continued)

2. Significant Accounting Policies (continued)

attributable to non-controlling interest of \$161,833,000 and \$163,594,000, respectively, representing the venture partners' interest in the equity and undistributed earnings of the consolidated ventures. For those ventures where the Health System does not maintain a controlling interest, the Health System accounts for its investment under the equity method of accounting.

Goodwill

The Health System records goodwill arising from a business combination as the excess of the purchase price and related costs over the fair value of the identifiable tangible and intangible assets acquired and liabilities assumed. As of June 30, 2024 and 2023, the Health System had goodwill of \$195,054,000 and \$187,298,000, respectively, that relates to the purchase of several entities, including purchases made by consolidated joint ventures. Goodwill is reflected in other assets.

Goodwill is tested annually for impairment at the reporting unit level. Impairment is the condition that exists when the carrying amount of goodwill exceeds its fair value. There was no impairment of goodwill in the twelve months ended June 30, 2024 or 2023.

Taxes

The Health System and certain subsidiaries are Texas nonprofit corporations exempt from federal taxation under IRC Section 501(a), but are required to pay income taxes for their unrelated business activities. Most of the income received by the Health System and its subsidiaries is exempt from taxation under IRC Section 501(a). The Health System also owns subsidiaries that are taxable. The overall impact of federal income taxes to the Health System's consolidated financial statements is not significant. The Health System additionally has certain subsidiaries that file partnership income tax returns for U.S. tax purposes, as well as filing franchise tax returns in the state of Texas. The statutes of limitations for tax years between 2020 and 2022 remain open in the major U.S. taxing jurisdictions in which the Health System and subsidiaries are subject to taxation.

The Health System follows the provisions of Accounting Standards Codification (ASC) 740, *Income Taxes*. Management has determined that there are no material uncertain tax positions that require recognition in the accompanying consolidated balance sheets. The Health System has net operating loss (NOL) tax carryforwards that generally will expire between 2024 and 2040, and certain NOLs that carry forward indefinitely, but are subject to an 80% limitation. No material deferred tax assets have been recorded due to the uncertainty the NOLs will be realized in the future. For the remaining subsidiaries, management is uncertain that the full amount of the NOLs will be realized in the future.

Memorial Hermann Health System

Notes to Consolidated Financial Statements (continued)

2. Significant Accounting Policies (continued)

Performance Indicator

The Health System's consolidated statements of operations and changes in net assets contain a performance indicator titled revenues in excess of expenses. Revenues in excess of expenses include the Health System's results from operations and non-operating activities and exclude changes in non-controlling interests, changes in unfunded pension obligations, restricted contributions and grants received, and certain other changes in net assets. Health System activities directly related to the furtherance of the Health System's purpose, as discussed in Note 1, are considered to be operating activities. Other activities that result in gains or losses are considered to be non-operating and primarily include investment losses and gains; gains and losses on extinguishment of debt; and other non-operating gains/losses, including unusual or infrequent recoveries or costs not directly related to operating activities.

New Accounting Pronouncement

In July 2016, the Financial Accounting Standards Board issued Accounting Standards Update (ASU) 2016-13, *Financial Instruments – Credit Losses (Topic 326): Measurement of Credit Losses on Financial Instruments*, which is intended to change how entities will measure credit losses for most financial assets and certain other instruments that are not measured at fair value through net income. The standard will replace today's "incurred loss" approach with an "expected loss" model for instruments measured at amortized cost and requires entities to record allowances for available-for-sale debt securities rather than reduce the carrying amount, as they do today under the other-than-temporary impairment model. It also simplifies the accounting model for purchased credit-impaired debt securities and loans. The provisions of ASU 2016-13 are effective for the Health System for the annual and interim periods starting July 1, 2023, and early adoption is permitted. The Health System adopted ASU 2016-13 in July 2023. There was no impact to the Health System's consolidated financial statements.

Memorial Hermann Health System

Notes to Consolidated Financial Statements (continued)

3. Community Service

In accordance with its purpose and values, the Health System is committed to providing high-quality, cost-effective health services to the community, including such underserved groups as the indigent and the elderly. Self-pay revenues are derived primarily from patients who do not have any form of health coverage. The revenues associated with self-pay patients are generally reported at the Health System's gross charges. The Health System evaluates these patients, after the patient's medical condition is determined to be stable, for their ability to pay based upon federal and state poverty guidelines, or qualifications for Medicaid or other governmental assistance programs, as well as the Health System's policy for charity care. The Health System provides care without charge to certain patients who qualify under the local charity care policy. The Health System's gross charity care charges include only services provided to patients who are unable to pay and qualify under the Health System's charity care policies. The Health System does not report a charity care patient's charges in revenues or in the provision for doubtful accounts, as it is the Health System's policy not to pursue collection of amounts related to these patients.

The Health System's management estimates its costs of care provided under its charity care programs utilizing a calculated ratio of costs to gross charges multiplied by the Health System's gross charges provided. For the years ended June 30, 2024 and 2023, these costs were \$373,469,000 and \$339,075,000, respectively.

The Health System operates emergency rooms at its hospitals that are open to the public 24 hours a day, 7 days a week. The Health System also operates Life Flight, an air ambulance service based at Memorial Hermann – Texas Medical Center (Level I trauma center); a burn unit; a transplant center; a Level I Pediatric Trauma facility; a Level II trauma center in the Woodlands; and one Level IV and one Level III neonatal intensive care unit. Furthermore, the Health System provides various community screenings for the detection of diseases and disorders, as well as a forum for various wellness activities and community health education classes.

In addition to the uncompensated care provided to patients, the Health System funds various community projects as part of its ongoing community benefit plan and commitment to Memorial Hermann Community Benefits Corp. (MHCBC). MHCBC's four driving pillars are: access to health care, emotional well-being, food as health, and exercise is medicine. The Health System collaborates with various community and city-wide task forces committed to building a healthier and more vital Houston community.

Memorial Hermann Health System

Notes to Consolidated Financial Statements (continued)

3. Community Service (continued)

Projects are developed in response to specific community needs in the Health System's service area identified through community needs assessments. Examples of projects funded include ten school-based health centers and three mobile dental vans serving more than 80 schools in five school districts; an ER Navigator program educating primary care patients about the importance of finding an appropriate medical home other than the emergency room; two neighborhood health centers serving the primary care needs of uninsured and underinsured patients; a nurse help-line providing nurse triage to greater Houston to educate and assist on the appropriate use of health resources; three Community Resource Centers helping improve overall health through assistance with health and social service connections, applications and education; and, collaborative work in a handful of at-risk communities predominantly providing exercise is medicine and food as health programming and providing financial support to primary care clinics and social service agencies that support the Health System.

As a part of its approval of the 1997 merger between Memorial Hospital System and Hermann Hospital Estate, a Harris County District Court entered an Agreed Order stipulating that Memorial Hermann – Texas Medical Center will continue providing charity care and community service in the amount of 6% of net revenue or \$22,500,000, whichever is greater. This amount is an additional 1% above the percentage required of all Texas nonprofit hospitals under the charity care provision of the Texas Health & Safety Code. During fiscal years 2024 and 2023, the Health System believes it has met all stipulations of the agreement. Revenues of the other Health System entities are not obligated under the agreement.

Memorial Hermann Health System

Notes to Consolidated Financial Statements (continued)

4. Investments and Assets Limited as to Use

Investments

The Health System maintains investments with various financial institutions, investment management firms, and direct equity investments and its policy is designed to limit exposure to any one institution or investment, therefore reducing overall investment risks. The following is a summary of unrestricted investments by classification:

	June 30	
	2024	2023
	<i>(In Thousands)</i>	
Cash and cash equivalents	\$ 17,398	\$ 3,859
Fixed income	358,144	358,661
Equity securities	1,487,073	1,366,951
Alternative investments:		
Hedge funds	962,418	928,510
Private equity	1,302,500	1,128,939
Real assets	594,461	557,532
Direct equity investments	228,891	143,012
Total investments	<u>\$ 4,950,885</u>	<u>\$ 4,487,464</u>

Memorial Hermann Health System

Notes to Consolidated Financial Statements (continued)

4. Investments and Assets Limited as to Use (continued)

Assets Limited as to Use

The following table sets forth the restricted purpose of the Health System's assets limited as to use:

	June 30	
	2024	2023
	<i>(In Thousands)</i>	
Bond indenture agreements	\$ 273,919	\$ 153,198
Self-insurance programs	141,925	113,256
Donor restrictions	160,071	136,882
Charity care, depreciation, and other funds	42,170	29,520
	618,085	432,856
Less current portion required for current liabilities	(147,000)	(165,542)
	\$ 471,085	\$ 267,314

Investment Income

Investment income related to net assets without donor restrictions is comprised of the following:

	Year Ended June 30	
	2024	2023
	<i>(In Thousands)</i>	
Interest and dividend income, net of fees	\$ 78,376	\$ 61,499
Realized gains, net	203,343	29,509
Unrealized gains, net	170,719	256,167
	\$ 452,438	\$ 347,175

Memorial Hermann Health System

Notes to Consolidated Financial Statements (continued)

5. Financial Assets and Liquidity Reserve

The Health System defines general expenditures as the expenditures related to operations of the Health System, excluding capital expenditures. The table below represents financial assets and liquidity resources available for general expenditures within one year. As part of the Health System's liquidity management plan, cash in excess of daily requirement is invested in money market funds and other liquid investments that can be drawn upon, if necessary, to meet liquidity needs. The Health System has assets limited as to use under self-insurance programs and or restricted by donors. These assets limited as to use are not reflected in the table below.

	June 30	
	2024	2023
	<i>(In Thousands)</i>	
Financial assets:		
Cash and cash equivalents	\$ 798,690	\$ 751,511
Investments	4,950,885	4,487,464
Patient accounts receivable	963,434	795,884
Other current assets	349,199	254,608
Total financial assets	7,062,208	6,289,467
Liquidity resource:		
Unused bank lines of credit	450,000	450,000
Less amounts not available within one year or not designated for general expenditures:		
Restricted funds included in cash	(113,517)	(136,333)
Committed capital expenditures	(145,599)	(171,827)
Alternative investments subject to redemption restrictions	(2,129,699)	(1,839,569)
Financial assets not available for use within one year	(2,388,815)	(2,147,729)
Financial assets and liquidity resources available for general expenditures within one year	<u>\$ 5,123,393</u>	<u>\$ 4,591,738</u>

The Health System executed an agreement for taxable commercial paper for \$150,000,000, effective July 31, 2024.

Memorial Hermann Health System

Notes to Consolidated Financial Statements (continued)

6. Property, Plant, and Equipment

Property, plant, and equipment consist of the following:

	June 30	
	2024	2023
	<i>(In Thousands)</i>	
Buildings and improvements	\$ 4,547,513	\$ 4,358,477
Building and equipment under finance lease	825,052	721,858
Equipment	2,741,331	2,789,201
Less accumulated depreciation	(5,207,388)	(5,059,476)
	<u>2,906,508</u>	<u>2,810,060</u>
Land	168,737	165,591
Construction-in-progress	374,612	203,738
	<u>\$ 3,449,857</u>	<u>\$ 3,179,389</u>

Under the provisions of ASC 840-40, *Leases – Sale-Leaseback Transactions*, and transition provisions of ASC 842, *Leases*, the Health System is required to continue to capitalize medical office buildings sold and leased back and to recognize an obligation for the sales proceeds received. As of June 30, 2023, the Health System had a deferred gain of \$149,218,000 due to specified forms of continuing involvement in the buildings. These medical office buildings are depreciated over the economic lives when placed in service. During fiscal year 2024, the Health System ceased to have continuing involvement in four office buildings and recognized \$62,978,000 of the deferred gain. Also during fiscal year 2024, the Health System elected to extend the leases for certain other office buildings that were subject to sale and leaseback transactions. As a result, a portion of the previously deferred gain on sale was reclassified as a lease obligation. As of June 30, 2024, the remaining deferred gain is \$22,608,000. As of June 30, 2024 and 2023, the following amounts are recorded in property, plant, and equipment, net and other long-term obligations:

	Property, Plant, and Equipment, Net		Other Accrued Expenses and Other Long-term Obligations	
	June 30		June 30	
	2024	2023	2024	2023
	<i>(In Thousands)</i>			
Medical office buildings and improvements	\$ 29,246	\$ 32,320	\$ 108,497	\$ 180,472

Memorial Hermann Health System

Notes to Consolidated Financial Statements (continued)

6. Property, Plant, and Equipment (continued)

As of June 30, 2024, future minimum payments related to these medical office buildings sold and leased back are as follows: \$15,910,000 in 2025, \$8,757,000 in 2026, \$8,730,000 in 2027, \$8,499,000 in 2028, \$8,669,000 in 2029 and \$55,778,000 thereafter.

7. Indebtedness

Long-term debt at June 30 consisted of the following:

	2024	2023
	<i>(In Thousands)</i>	
Obligations issued under the Memorial Hermann Health Master Trust Indenture:		
Revenue bonds, in variable rate demand mode, with interest rates at 3.50% at June 30, 2024, due in varying installments through July 1, 2055:		
Series 2014C and D	\$ —	\$ 70,400
Series 2016B, C, and D	—	253,975
Series 2019E and F	—	132,800
Series 2024D, E, and F	333,045	—
Revenue bonds, in variable rate mode, with interest rate at 4.73% at June 30, 2024, due in annual installments through December 1, 2049:		
Series 2013B	—	11,370
Series 2019C	—	51,570
Series 2022C	52,045	52,045
Revenue bonds, in fixed-rate mode, bearing interest from 2.30% to 5%, due in annual installments through July 1, 2054:		
Series 2014A	78,785	80,660
Series 2016A	124,015	124,015
Series 2019A	118,595	135,200
Series 2019B	57,880	115,760
Series 2020	—	5,425
Series 2020A	30,110	30,110
Series 2020B	8,230	8,230
Series 2020C	68,300	102,445
Series 2020D	91,425	91,425
Series 2022A	189,235	189,235
Series 2022B	54,740	54,740
Series 2022D	49,000	49,000
Series 2022E	8,160	17,355
Series 2024B	427,480	—
Series 2024C	135,785	—
Other long-term debt obligations:		
Finance lease obligations	833,075	734,639
Other notes	59,165	17,074
	2,719,070	2,327,473
Premiums on long-term debt, net	107,639	70,525
Less deferred financing fees	(8,195)	(8,554)
	2,818,514	2,389,444
Less current portion, including amounts subject to self-liquidity	(410,236)	(520,870)
	<u>\$ 2,408,278</u>	<u>\$ 1,868,574</u>

Memorial Hermann Health System

Notes to Consolidated Financial Statements (continued)

7. Indebtedness (continued)

Debt Obligations

The Health System has issued revenue bonds through the Harris County Health Facilities Development Corporation and the Harris County Education Facilities Finance Corporation. Payments to the bondholders are funded by the issuing affiliates under a master trust indenture with the trustees of the respective bond issues. The Health System and substantially all operating affiliates have agreed to arrangements and indentures related to the bonds to abide by guidelines regarding repayment, financial performance, organizational changes, reporting, and additional borrowing. Certain variable rate demand bonds were issued under the Health System's self-liquidity program and, therefore, have been classified as current liabilities. The Health System will provide its own liquidity to purchase any tendered bonds that cannot be successfully remarketed.

In September 2021, the Health System entered into a new syndicated revolving line of credit agreement for a term of three years with a maximum amount available of \$450,000,000. The line of credit may be used for general corporate purposes and for the commercial paper bond program's self-liquidity, if needed. On August 2, 2024, the Health System entered an amended and restated syndicated revolving line of credit for a term of three years with a maximum amount available of \$450,000,000. No funds have been drawn on the line of credit as of June 30, 2024 and 2023.

On June 27, 2024, the Health System issued the Series 2024B (\$427,480,000) fixed rate bonds, Series 2024C (\$135,785,000) fixed rate put bonds, Series 2024D (\$111,015,000) weekly variable rate demand bonds, Series 2024E (\$111,015,000) weekly variable rate demand bonds, and Series 2024F (\$111,015,000) weekly variable rate demand bonds. The Series 2024D, E, and F Series bonds were issued under the Health System's self-liquidity program and, therefore, have been classified as current liabilities. The 2014C and D; 2016C, D; 2019C-2; 2019E; and F Series were redeemed by the Series 2024 bonds. The Health System deposited funds into an escrow in June 2024 with proceeds from 2024C and E for an irrevocable provision to call Series 2016B, 2019B-2, and 2020C-2 for redemption, resulting in these bonds being recorded as a legal defeasance in fiscal year 2024. The redemption will take place on September 24, 2024.

On July 31, 2024, the Health System established a taxable commercial paper program not to exceed a principal amount of \$150,000,000. No funds have been drawn on the commercial paper as of September 25, 2024.

Memorial Hermann Health System

Notes to Consolidated Financial Statements (continued)

7. Indebtedness (continued)

Effective July 1, 2022, the Health System and Access Clinical Holdings of Texas, LLC (ACHT) began operations of a joint venture, Memorial Hermann-GoHealth Urgent Care, LLC. The Health System and Access Clinical Partners, LLC (parent of ACHT) are jointly and severally liable for the joint venture bank debt of \$112,000,000, of which \$73,800,000 was outstanding on June 30, 2024.

Other notes consist of secured loans by financial institutions to finance the equipment purchases of the Health System's joint venture partnerships. As of June 30, 2024, scheduled principal payments for outstanding debt (excluding finance leases and other notes) for the next five fiscal years are as follows: \$45,320,000 in 2025, \$48,078,000 in 2026, \$49,913,000 in 2027, \$39,249,000 in 2028, and \$42,121,000 in 2029.

Leases

The Health System leases certain buildings, medical offices, and various equipment types. The Health System determines if an arrangement is a lease at inception. Operating leases are included in operating lease right-of-use assets and current and long-term operating lease right-of-use obligations on the consolidated balance sheets. Finance leases are included in property and equipment, current portion of long-term debt, and long-term debt on the consolidated balance sheets. Right-of-use assets and lease liabilities are recognized based on the net present value of the future minimum lease payments over the lease term at commencement date. The Health System has agreements with lease and non-lease components. The non-lease components are excluded from the right-of-use assets and liabilities. The Health System uses a risk-free rate in measuring lease liabilities and lease terms range from 1 to 24 years.

Memorial Hermann Health System

Notes to Consolidated Financial Statements (continued)

7. Indebtedness (continued)

As of June 30, 2024, minimum future rentals under non-cancelable leases, for the next five fiscal years and in aggregate, are as follows:

	Finance Leases	Operating Leases
	<i>(In Thousands)</i>	
2025	\$ 73,252	\$ 86,691
2026	78,609	86,184
2027	75,413	82,970
2028	73,804	70,886
2029	71,798	63,671
Thereafter	1,112,300	168,529
Total minimum lease payments	1,485,176	558,931
Less portion representing interest	(652,101)	(60,076)
Total lease obligations	<u>\$ 833,075</u>	<u>\$ 498,855</u>

For the years ended June 30, 2024 and 2023, lease cost for operating leases was \$89,320,000 and \$88,757,000, respectively; cash paid for operating leases was \$88,249,000 and \$87,094,000, respectively. For operating and finance lease assets and liabilities, the table below presents the weighted average remaining lease term and discount rate as of the years ended June 30, 2024 and 2023. For the years ended June 30, 2024 and 2023, lease cost for finance leases was \$81,784,000 and \$75,542,000, respectively; cash paid for financing leases was \$62,480,000 and \$58,571,000, respectively.

	June 30 2024	2023
Weighted average remaining lease term (years):		
Operating leases	7.62	7.37
Finance leases	20.46	20.31
Weighted average discount rate:		
Operating leases	2.81%	2.24%
Finance leases	5.81%	5.84%

For the years ended June 30, 2024 and 2023, variable lease expense was \$17,705,000 and \$15,678,000, respectively.

Memorial Hermann Health System

Notes to Consolidated Financial Statements (continued)

8. Interest Rate Swap Agreements

The Health System utilizes interest rate swap agreements to manage its capital costs and interest rate risk. The following table summarizes the Health System's swap portfolio, the fair values at June 30, 2024 and 2023, the change in value, and the net amounts paid and received for the years ended June 30, 2024 and 2023 (in thousands):

Swap Description	Term Date	Interest Rate Agreements	Aggregate Notional Amount	Fair Value Asset (Liability)		Change in Fair Value		Settlement Activity Paid (Received)	
				June 30		June 30		Year Ended June 30	
				2024	2023	2024	2023	2024	2023
SOFR-based to fixed (5.343%)	2032	1	\$ 113,120	\$ (8,216)	\$ (11,296)	\$ 3,080	\$ 9,615	\$ (7)	\$ 1,611
SOFR-based to fixed (1.90%)	2041	1	132,325	25,390	15,789	9,601	10,009	(2,634)	(943)
SOFR-based to fixed (3.685%)	2027	1	27,000	(192)	(338)	146	1,134	(2)	329
SOFR-based to fixed (3.690%)	2027	1	14,800	(136)	(241)	105	880	(1)	237
SOFR-based to fixed (3.685%)	2027	1	19,300	(181)	(319)	138	1,158	(2)	308
SOFR-based constant maturity swap	2034	1	500,000	(3,339)	(10,577)	7,238	(9,682)	6,286	3,280
		6	\$ 806,545	\$ 13,326	\$ (6,982)	\$ 20,308	\$ 13,114	\$ 3,640	\$ 4,822

The notional amounts under each of the interest rate swap agreements are reduced in conjunction with the Health System's principal payments on the associated bonds, except for the constant maturity swap, the notional amount of which does not change. At June 30, 2024 and 2023, the fair value of swap agreements was an asset of \$13,326,000 and a liability of \$6,982,000, respectively, and has been included in other assets and other long-term liabilities on the accompanying consolidated balance sheets. As of June 30, 2024, none of the Health System's swap agreements include provisions that would require posting of collateral.

The Health System classified the net interest cost on its interest rate swaps for the years ended June 30, 2024 and 2023 of \$3,640,000 and \$4,822,000, respectively, in non-operating expenses (interest rate swap agreements) on the consolidated statements of operations and changes in net assets.

Memorial Hermann Health System

Notes to Consolidated Financial Statements (continued)

9. Fair Value Measurements

The Health System categorizes, for disclosure purposes, assets and liabilities measured at fair value in the consolidated financial statements based upon whether the inputs used to determine their fair values are observable or unobservable. Observable inputs are inputs that are based on market data obtained from sources independent of the reporting entity. Unobservable inputs are inputs that reflect the reporting entity's own assumptions about pricing the asset or liability, based on the best information available in the circumstances.

In certain cases, the inputs used to measure fair value may fall into different levels of the fair value hierarchy. In such cases, an asset's or liability's level within the fair value hierarchy is based on the lowest level of input that is significant to the fair value measurement of the asset or liability. The Health System's assessment of the significance of a particular input to the fair value measurement in its entirety requires judgment and considers factors specific to the asset or liability.

The Health System follows the three-level fair value hierarchy to categorize these assets and liabilities recognized at fair value at each reporting period, which prioritizes the inputs used to measure such fair values. Level inputs are defined as follows:

Level 1 – Quoted prices (unadjusted) in active markets for identical assets or liabilities on the reporting date.

Level 2 – Inputs to the valuation methodology other than quoted market prices included in Level 1 that are observable for the asset or liability. Level 2 pricing inputs include quoted prices for similar assets and liabilities in active markets, and inputs that are observable for the asset or liability, either directly or indirectly, for substantially the full term of the financial instrument.

Level 3 – Inputs that are unobservable for the asset or liability.

The following tables present financial instruments carried at fair value as of June 30, 2024 and 2023 on a recurring basis. The tables do not include contributions receivable of \$22,025,000 and \$27,197,000 at June 30, 2024 and 2023, respectively, and real estate of \$187,000 that are not carried at fair value and are included in assets limited as to use on the consolidated balance sheets. For alternative investments presented at net asset value (NAV) in the table below, the valuations provided by the respective investment's management consider variables, such as the financial performance of underlying investments, recent sales prices of underlying investments, and other

Memorial Hermann Health System

Notes to Consolidated Financial Statements (continued)

9. Fair Value Measurements (continued)

pertinent information. In addition, actual market exchanges at year-end provide additional observable market inputs of the exit price. The majority of these funds have restrictions on the timing of withdrawals, which may reduce liquidity, in some cases over 12 months. Direct equity investments are investments in private companies and are valued at cost, less impairment.

	June 30, 2024			
	Level 1	Level 2	Level 3	Total
	<i>(In Thousands)</i>			
Assets				
Investments:				
Cash and equivalents	\$ 17,398	\$ –	\$ –	\$ 17,398
U.S. Government securities	–	89,772	–	89,772
Pooled funds:				
Domestic equities	108,592	192,846	–	301,438
Global equities	490,706	4,043	–	494,749
Fixed income	244,719	–	–	244,719
Hedge funds	75,305	403,126	–	478,431
Corporate obligation	8,405	15,248	–	23,653
Corporate equities	417,447	–	–	417,447
Global equities	273,439	–	–	273,439
Assets limited as to use:				
Cash and equivalents	194,121	–	–	194,121
Pooled funds:				
Domestic equities	12,287	11,081	–	23,368
Global equities	83,214	–	–	83,214
Fixed income	59,894	–	–	59,894
Hedge funds	1,986	23,596	–	25,582
Corporate equities	28,971	–	–	28,971
Global equities	15,097	–	–	15,097
Assets measured at fair value	2,031,581	739,712	–	2,771,293
Assets at NAV:				
Pooled funds:				
Hedge fund				501,322
Private equity				1,303,791
Real assets				594,461
Direct equity investments				228,891
Interest rate swap agreements	–	13,326	–	13,326
Total assets	\$ 2,031,581	\$ 753,038	\$ –	\$ 5,413,084

Memorial Hermann Health System

Notes to Consolidated Financial Statements (continued)

9. Fair Value Measurements (continued)

	June 30, 2023			
	Level 1	Level 2	Level 3	Total
	(In Thousands)			
Assets				
Investments:				
Cash and equivalents	\$ 3,859	\$ —	\$ —	\$ 3,859
U.S. Government securities	—	105,640	—	105,640
Pooled funds:				
Domestic equities	270,134	100,651	—	370,785
Global equities	436,116	280,343	—	716,459
Fixed income	240,101	—	—	240,101
Hedge funds	26,621	425,834	—	452,455
Corporate obligation	—	12,920	—	12,920
Corporate equities	66,519	—	—	66,519
Global equities	213,188	—	—	213,188
Assets limited as to use:				
Cash and equivalents	42,004	—	—	42,004
Pooled funds:				
Domestic equities	19,031	—	—	19,031
Global equities	72,869	4,735	—	77,604
Fixed income	47,040	—	—	47,040
Hedge funds	—	19,669	—	19,669
Corporate equities	5,351	—	—	5,351
Global equities	13,953	—	—	13,953
Assets measured at fair value	1,456,786	949,792	—	2,406,578
Assets at NAV:				
Pooled funds:				
Hedge fund				491,333
Private equity				1,128,939
Real assets				557,532
Direct equity investments				143,012
Total assets	\$ 1,456,786	\$ 949,792	\$ —	\$ 4,727,394
Liabilities				
Interest rate swap agreements	\$ —	\$ 6,982	\$ —	\$ 6,982
Total liabilities	\$ —	\$ 6,982	\$ —	\$ 6,982

Memorial Hermann Health System

Notes to Consolidated Financial Statements (continued)

9. Fair Value Measurements (continued)

The fair values of the securities included in Level 1 were determined through quoted market prices, and include money market funds, mutual funds, and marketable debt and equity securities. The fair values of Level 2 securities were determined through evaluated bid prices based on recent trading activity, and other relevant information, including market interest rate curves and referenced credit spreads, and estimated prepayment rates, where applicable, are used for valuation purposes and are provided by third-party services where quoted market values are not available. The fair values of the interest rate swap agreements included in Level 2 were determined using third-party models that use as their inputs observable market conditions.

10. Pension Plans

The Health System sponsors a cash balance defined benefit pension plan covering all eligible employees. The Health System's funding policy is to contribute amounts to the plan sufficient to meet the minimum funding requirements set forth in the Employee Retirement Income Security Act of 1974, plus such additional amounts as the Health System may determine to be appropriate from time to time. Funding requirements are determined through consultation with an independent actuary. Effective July 1, 2011, the Health System closed the plan to new participants. Participants as of June 30, 2011 continue to accrue benefits.

The Health System recognizes the funded status (that is, the difference between the fair value of plan assets and the projected benefit obligations) of its plan on the consolidated balance sheets, with a corresponding adjustment to net assets. Actuarial gains and losses that arise and are not recognized as net periodic pension cost in the same periods are recognized as a component of net assets.

The assumptions used in calculating the pension amounts recognized in the Health System's consolidated financial statements include discount rates, interest cost, expected return on plan assets, retirement and mortality rates, inflation rates, salary growth, and other factors. While the Health System believes the assumptions used are appropriate, differences in actual experience or changes in assumptions may affect future pension obligations and expense.

The following tables set forth the plan's funding status as of the measurement dates, amounts recognized in the consolidated financial statements, and assumptions used. The plan's measurement dates were June 30, 2024 and 2023, with participant census dates of July 1, 2023 and 2022.

Memorial Hermann Health System

Notes to Consolidated Financial Statements (continued)

10. Pension Plans (continued)

	Year Ended June 30	
	2024	2023
	<i>(In Thousands)</i>	
Change in projected benefit obligation:		
Benefit obligation, beginning of year	\$ 620,796	\$ 590,401
Service cost	19,683	19,262
Interest cost	32,570	27,836
Actuarial (gain) loss, net	(13,103)	21,866
Benefits paid	(38,937)	(35,767)
Administrative expenses paid	(2,698)	(2,802)
Projected benefit obligation (including \$613,560 and \$615,668 in 2024 and 2023, respectively, in accumulated benefit obligation), end of year	<u>\$ 618,311</u>	<u>\$ 620,796</u>
Change in plan assets:		
Fair value of assets, beginning of year	\$ 624,277	\$ 620,700
Employer contributions	11,195	–
Return on plan assets	44,512	42,146
Benefits paid	(38,937)	(35,767)
Administrative expenses paid	(2,698)	(2,802)
Fair value of assets, end of year	<u>\$ 638,349</u>	<u>\$ 624,277</u>
Funded status:		
Pension asset recorded in other assets on the consolidated balance sheets	<u>\$ 20,038</u>	<u>\$ 3,481</u>

Memorial Hermann Health System

Notes to Consolidated Financial Statements (continued)

10. Pension Plans (continued)

	Year Ended June 30	
	2024	2023
	<i>(In Thousands)</i>	
Components of net periodic pension cost:		
Service cost	\$ 19,683	\$ 19,262
Interest cost	32,570	27,836
Expected return on plan assets	(43,908)	(33,874)
Amortization of prior service cost	(7,140)	(7,140)
Amortization of losses and other, net	5,333	2,452
Net periodic pension cost	<u>\$ 6,538</u>	<u>\$ 8,536</u>

Service cost is included in salaries, benefits, and related personnel costs in the consolidated financial statements and other costs are included in investment gains and other, net.

Weighted average assumptions for determining benefit obligations at end of year and net periodic pension cost for the year:

Discount rates – benefit obligations	5.66%	5.55%
Discount rates – net periodic pension cost	5.55	4.94
Rates of increase in future compensation levels	3.02	3.05
Expected long-term rate of return on plan assets	7.30	5.50

The assumption for the expected return on assets is derived from a study conducted by the Health System's actuaries and financial management. The study includes a review of the plan's asset allocation strategy, anticipated long-term performance of individual asset classes, risks and correlations of asset classes, and general economic conditions of the investment marketplace. Because of revisions to the plan's investment policy, asset allocation strategy, and changes in professional managers utilized, historical returns performance of the plan was not a significant factor in the analysis.

Memorial Hermann Health System

Notes to Consolidated Financial Statements (continued)

10. Pension Plans (continued)

The assets of the pension plan by weighted average asset allocation categories are set forth in the following table:

	2024	2023
Asset category:		
Short-term investments	2%	2%
Fixed income	26	26
Domestic equity	8	5
Global equity	6	8
International equity	7	8
Risk parity	0	1
Private equity and hedge funds	51	50
Total	100%	100%

The plan's general investment objective is to seek to achieve attractive long-term total return from income and growth of capital over a full market cycle with a low-to-moderate level of risk, emphasizing, primarily, the preservation of principal and, secondarily, the real purchasing power of assets over the long term, while maintaining adequate liquidity to meet benefits and expense cash flow requirements when due through the utilization of investments in a balanced and diversified portfolio of equity, fixed income, short-term liquid securities, and alternative investments. Investments will be well-diversified across individual issuers, economic sectors, and asset classes to ensure a level of risk that is comparable to the general investment markets. For the fiscal year ended June 30, 2024, the targeted allocation by strategy for the investments is cash 2%, credit 46% and growth 52%. At June 30, 2024, the unfunded commitment related to the plan's assets is \$61,316,000.

Memorial Hermann Health System

Notes to Consolidated Financial Statements (continued)

10. Pension Plans (continued)

	June 30, 2024			
	Level 1	Level 2	Level 3	Total
	<i>(In Thousands)</i>			
Cash and cash equivalents	\$ 16,771	\$ –	\$ –	\$ 16,771
U.S. Government securities	–	14,686	–	14,686
Pooled funds:				
Domestic equities	–	20,462	–	20,462
Global equities	52,837	866	–	53,703
Fixed income	100,019	–	–	100,019
Hedge fund	585	57,326	–	57,911
Corporate obligations	–	51,079	–	51,079
Corporate equities	32,412	–	–	32,412
Global equities	24,819	–	–	24,819
Assets measured at fair value	227,443	144,419	–	371,862
Assets at NAV:				
Pooled funds:				
Hedge funds				61,528
Private equity				128,767
Real assets				76,192
Total assets	\$ 227,443	\$ 144,419	\$ –	\$ 638,349

Memorial Hermann Health System

Notes to Consolidated Financial Statements (continued)

10. Pension Plans (continued)

June 30, 2023				
	Level 1	Level 2	Level 3	Total
<i>(In Thousands)</i>				
Cash and cash equivalents	\$ 12,749	\$ –	\$ –	\$ 12,749
U.S. Government securities	–	10,539	–	10,539
Pooled funds:				
Domestic equities	18,948	10,896	–	29,844
Global equities	39,587	29,460	–	69,047
Fixed income	94,087	–	–	94,087
Hedge fund	6,892	57,205	–	64,097
Corporate obligations	–	60,368	–	60,368
Corporate equities	8,520	–	–	8,520
Global equities	28,561	–	–	28,561
Assets measured at fair value	209,344	168,468	–	377,812
Assets at NAV:				
Pooled funds:				
Hedge funds				63,704
Private equity				116,521
Real assets				66,240
Total assets	\$ 209,344	\$ 168,468	\$ –	\$ 624,277

As described in Note 9, management has elected to use the net asset value per share, or its equivalent, as a practical expedient for fair value of the plan's interest in alternative investments. The majority of these funds have restrictions on the timing of withdrawals, which may reduce liquidity, in some cases over 12 months.

All of the assets are managed by external investment managers and are maintained in actively managed security portfolios. Derivative investments are not allowed, except by fund managers who employ such techniques to offset or reduce the risk associated with an existing group of investments. There were no required contributions for fiscal year 2024.

Memorial Hermann Health System

Notes to Consolidated Financial Statements (continued)

10. Pension Plans (continued)

	Estimated <i>(In Thousands)</i>
Expected benefit payments estimated for fiscal years:	
2025	\$ 51,133,000
2026	49,962,000
2027	48,769,000
2028	48,650,000
2029	49,091,000
2030 and thereafter	245,743,000

In addition to the defined benefit plan discussed above, eligible employees participate in certain other retirement plans, including defined contribution plans with employer matches of \$119,806,000 and \$107,800,000 for 2024 and 2023, respectively.

11. Self-Funded Liabilities

The Health System is self-insured for general and professional liability, errors and omissions, and workers' compensation claims, and maintains excess insurance coverage at varying levels. A provision is made for estimated losses and related expenses on risks not covered by insurance. The provision includes estimated amounts for asserted claims, reported incidents for which a claim has not been asserted, and claims incurred but not reported. The provision is based on specific claim loss estimates by the Health System's management and on estimates of total annual losses by an independent consulting actuary using the Health System and similar facility experience.

The Health Professionals Insurance Company, Ltd. (HePIC), a wholly owned subsidiary, is a captive insurance company that provides professional liability, general liability, and other insurance coverage for the Health System's affiliates. The Health System funds HePIC's required insurance reserves. Funding amounts are based on actuarial recommendations. The assets of HePIC and the established liability for self-funded losses are reported on the consolidated balance sheets. Investment income from the assets and the provision for estimated self-funded losses and administrative costs are reported on the consolidated statements of operations and changes in net assets. The Health System's established liability for self-funded losses was \$89,655,000 and \$68,015,000 as of June 30, 2024 and 2023, respectively, and is recorded in accrued payroll and related expenses, other accrued expenses, and other long-term obligations on the accompanying consolidated balance sheets.

Memorial Hermann Health System

Notes to Consolidated Financial Statements (continued)

12. Functional Expenses

The table below present expenses by both their nature and function for the years ended June 30, 2024 and 2023:

	Health Services	Insurance Services	General and Administrative	Total
Year ended June 30, 2024:				
Salaries, benefits, and related personnel costs	\$ 3,538,970	\$ 1,089	\$ 142,130	\$ 3,682,189
Services and other	2,429,271	119,771	95,011	2,644,053
Supplies and medicines	1,503,320	3,894	5,023	1,512,237
Depreciation and amortization	336,306	730	48,938	385,974
Interest	111,461	—	—	111,461
	<u>\$ 7,919,328</u>	<u>\$ 125,484</u>	<u>\$ 291,102</u>	<u>\$ 8,335,914</u>
Year ended June 30, 2023:				
Salaries, benefits, and related personnel costs	\$ 3,272,603	\$ 1,701	\$ 133,401	\$ 3,407,705
Services and other	2,044,571	95,118	92,719	2,232,408
Supplies and medicines	1,417,828	3,489	5,972	1,427,289
Depreciation and amortization	326,363	833	41,609	368,805
Interest	110,973	—	—	110,973
	<u>\$ 7,172,338</u>	<u>\$ 101,141</u>	<u>\$ 273,701</u>	<u>\$ 7,547,180</u>

13. Commitments and Contingencies

Litigation

From time to time, the Health System is subject to litigation in the ordinary course of its operations. In management's opinion, any future settlements or judgments on asserted or unasserted claims will not have a material effect on the Health System's consolidated financial position.

Various federal and state agencies have initiated investigations regarding reimbursement claimed by the Health System and other matters. The investigations are in various stages of discovery, and the ultimate resolution of these matters, including the liabilities, if any, cannot be readily determined; however, in the opinion of management, the results of these investigations will not have a material adverse impact on the consolidated financial statements of the Health System.

Memorial Hermann Health System

Notes to Consolidated Financial Statements (continued)

13. Commitments and Contingencies (continued)

Other

Under terms of an agreement, as amended, originally dated January 1, 1968, and superseded with a new agreement dated August 16, 2004, the Health System and the University of Texas Health Science Center at Houston (the University) affiliated to operate and maintain a patient care, medical teaching, research, and community service facility. The agreement specifies that the Health System will serve as the primary private hospital teaching site for the University, and operate and maintain a fully accredited hospital, while maintaining final authority over operational policy. The University agrees to offer the hospital the opportunity to accommodate all teaching programs and clinical programs, maintain fully accredited educational programs, and conduct research activities while utilizing the Health System.

Mutual commitments include administrative appointments and sharing of certain operational and research costs. Expenses for obligations to the University for the years ended June 30, 2024 and 2023 totaled \$643,000,000 and \$551,000,000, respectively. This agreement was renewed on October 15, 2019 for an additional ten years.

Supplementary Information

Memorial Hermann Health System
Schedule of Expenditures of Federal Awards
Year ended June 30, 2024

Federal Grantor/Pass-Through Grantor/Program Title	Federal Assistance Listing Number	Federal Award Identifying Number	Pass- Through Award Identifying Number	Federal Award Expenditures	Passed Through to Subrecipients
U.S. Department of Health and Human Services					
Direct Agency Funded Programs:					
ACL National Institute on Disability, Independent Living, and Rehabilitation Research *	93.433	90DPAD0010	—	\$ 1,008,820	\$ 202,329
ACL National Institute on Disability, Independent Living, and Rehabilitation Research *	93.433	90DPTB0016	—	40,342	—
ACL National Institute on Disability, Independent Living, and Rehabilitation Research *	93.433	90DPAD0001	—	207,511	140,774
ACL National Institute on Disability, Independent Living, and Rehabilitation Research *	93.433	90DPTB0025	—	409,255	58,678
ACL National Institute on Disability, Independent Living, and Rehabilitation Research *	93.433	90SIMS0019	—	522,464	34,489
ACL National Institute on Disability, Independent Living, and Rehabilitation Research *	93.433	90DPAD0013	—	165,257	132,830
				<u>2,353,649</u>	<u>569,100</u>
Substance Abuse and Mental Health Services Projects of Regional and National Significance	93.243	H79SM084831	—	540,142	—
ACL Independent Living State Grants	93.369	90ISTA0002	—	274,596	67,206
ACL Centers for Independent Living	93.432	90ILTA0002	—	991,743	395,420
Total Direct Agency Funded Programs				\$ 4,160,130	\$ 1,031,726
Passed-Through Programs:					
<i>Passed-Through US Aging:</i>					
Special Programs for the Aging, Title IV, and Title II, Discretionary Projects *	93.048	90NWCH0001	—	\$ 4,457	\$ —
COVID-19 - Special Programs for the Aging, Title IV, and Title II, Discretionary Projects *	93.048	90EEC50001	—	21,630	—
COVID-19 - Special Programs for the Aging, Title IV, and Title II, Discretionary Projects *	93.048	90HDRC0007	—	312,064	—
				<u>338,151</u>	<u>—</u>

Memorial Hermann Health System
Schedule of Expenditures of Federal Awards
Year ended June 30, 2024

Federal Grantor/Pass-Through Grantor/Program Title	Federal Assistance Listing Number	Federal Award Identifying Number	Pass-Through Award Identifying Number	Federal Award Expenditures	Passed Through to Subrecipients
<i>Passed-Through Indiana University:</i>					
ACL National Institute on Disability, Independent Living, and Rehabilitation Research *	93.433	90DPHF0006	—	21,171	—
ACL National Institute on Disability, Independent Living, and Rehabilitation Research *	93.433	90DPTB0035	9825	23,179	—
ACL National Institute on Disability, Independent Living, and Rehabilitation Research *	93.433	90DPTB0035	9927	82	—
				<u>44,432</u>	<u>—</u>
<i>Passed-Through University of Houston:</i>					
ACL National Institute on Disability, Independent Living, and Rehabilitation Research *	93.433	90IFST0006	R-22-0078	32,679	—
<i>Passed-Through University of Delaware:</i>					
ACL National Institute on Disability, Independent Living, and Rehabilitation Research *	93.433	90DPCP0010	UDR0000137	78,176	36,077
<i>Passed-Through Research Foundation of CUNY on behalf of Hunter College:</i>					
ACL National Institute on Disability, Independent Living, and Rehabilitation Research *	93.433	90DPGE0013	M00003839-03/04	100,076	—
				<u>255,363</u>	<u>36,077</u>
<i>Passed-Through University of Delaware:</i>					
Nursing Research *	93.361	R01NR018684	55381	58,770	—
<i>Passed-Through Baylor College of Medicine:</i>					
Nursing Research *	93.361	5R01NR013658	—	60,279	—
				<u>119,049</u>	<u>—</u>
<i>Passed-Through University of Cincinnati:</i>					
Extramural Research Programs in the Neurosciences and Neurological Disorders *	93.853	U01NS110772	012765-140423	47,716	—
Extramural Research Programs in the Neurosciences and Neurological Disorders *	93.853	U01NS110772	012765-00025		
		U01NS110772	012765-00032	55,056	—
				<u>102,772</u>	<u>—</u>

Memorial Hermann Health System
Schedule of Expenditures of Federal Awards
Year ended June 30, 2024

Federal Grantor/Pass-Through Grantor/Program Title	Federal Assistance Listing Number	Federal Award Identifying Number	Pass-Through Award Identifying Number	Federal Award Expenditures	Passed Through to Subrecipients
<i>Passed-Through The Ohio State University:</i>					
Extramural Research Programs in the Neurosciences and Neurological Disorders *	93.853	UG3NS117844	GR128734 I SPC-1000006866 GR133232 I SPC-1000012424	117,321	—
<i>Passed-Through University of Michigan:</i>					
Extramural Research Programs in the Neurosciences and Neurological Disorders *	93.853	U01NS099046	SUBK00015080	22,997	—
<i>Passed-Through Northwestern University:</i>					
Extramural Research Programs in the Neurosciences and Neurological Disorders *	93.853	R01NS110779	60052911 MHHS	13,494	—
<i>Passed-Through Washington University:</i>					
Extramural Research Programs in the Neurosciences and Neurological Disorders *	93.853	U01NS100699	WU-22-0134-MOD-4 WU-22-0134-MOD-5	33,776	—
				290,360	—
<i>Passed-Through University of Texas Health Science Center, Houston:</i>					
Research and Training in Complementary and Alternative Medicine *	93.213	R34AT011530	SA0002872	24,813	—
<i>Passed-Through American Association on Health and Disability:</i>					
Trans-NIH Research Support *	93.310	OT2OD031916	202306	3,109	—
<i>Passed Through CDC Foundation:</i>					
<i>Passed-Through Able South Carolina:</i>					
Strengthening Public Health Systems and Services through National Partnerships to Improve and Protect the Nation's Health *	93.421	NU38OT000288	—	50,504	—
Total Passed-Through Programs				\$ 1,081,349	\$ 36,077
Total U.S. Department of Health and Human Services				\$ 5,241,479	\$ 1,067,803

Memorial Hermann Health System
Schedule of Expenditures of Federal Awards
Year ended June 30, 2024

Federal Grantor/Pass-Through Grantor/Program Title	Federal Assistance Listing Number	Federal Award Identifying Number	Pass- Through Award Identifying Number	Federal Award Expenditures	Passed Through to Subrecipients
U.S. Department of Defense					
Direct Agency Funded Programs:					
Military Medical Research and Development *	12.420	HT94252310567	—	\$ 218,070	\$ 158,049
Military Medical Research and Development *	12.420	HT94252310396	—	126,350	19,017
Total Direct Agency Funded Programs				\$ 344,420	\$ 177,066
Passed-Through Programs:					
<i>Passed through Baylor College of Medicine:</i>					
Military Medical Research and Development *	12.420	W81XWH1910735	—	\$ 1,559	\$ —
<i>Passed through Icahn School of Medicine at Mount Sinai:</i>					
Military Medical Research and Development *	12.420	W81XWH2210999	0258-A921-4605	1,791	—
<i>Passed through University of Texas Health Science Center:</i>					
Military Medical Research and Development *	12.420	W81XWH2210848	SA0002161	55,917	—
				59,267	—
<i>Passed through Medical Technology Enterprise Consortium</i>					
<i>Passed through University of California, San Francisco:</i>					
Basic Scientific Research *	12.431	—	A131954	1,253	—
Total Passed-Through Programs				\$ 60,520	\$ —
Total U.S. Department of Defense				\$ 404,940	\$ 177,066

Memorial Hermann Health System
Schedule of Expenditures of Federal Awards
Year ended June 30, 2024

Federal Grantor/Pass-Through Grantor/Program Title	Federal Assistance Listing Number	Federal Award Identifying Number	Pass- Through Award Identifying Number	Federal Award Expenditures	Passed Through to Subrecipients
U.S. Department of Homeland Security					
<i>Passed through Texas Division of Emergency Management:</i>					
COVID-19 - Disaster Grants – Public Assistance (Presidentially Declared Disasters)	97.036	4485DRTXP0000 001	–	\$ 97,202,454	\$ –
Total Passed-Through Programs				\$ 97,202,454	\$ –
Total U.S. Department of Homeland Security				\$ 97,202,454	\$ –
Total Federal Expenditures				\$102,848,873	\$ 1,244,869

Cluster Title:

* Represents Research and
Development Cluster

Total Expenditures:

R&D cluster – \$3,839,938

ALN 12.420 – \$403,687

ALN 93.433 – \$2,609,012

Memorial Hermann Health System

Notes to the Schedule of Expenditures of Federal Awards

Year Ended June 30, 2024

1. Basis of Presentation

The Schedule presents federal grant activities of the System and is reported on the accrual basis of accounting, in accordance with the generally accepted accounting principles in the United States of America. The information in the Schedule is presented in accordance with the requirements of Title 2 U.S. *Code of Federal Regulations* (CFR) Part 200, *Uniform Cost Principles, and Audit Requirements for Federal Awards (Uniform Guidance)*.

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MEMORIAL HERMANN HEALTH SYSTEM

**Independent Auditor's Report
in accordance with the
Uniform Guidance**

Year Ended June 30, 2024

MEMORIAL HERMANN HEALTH SYSTEM
YEAR ENDED JUNE 30, 2024
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**Report of Independent Auditors on Compliance for Each Major Federal Program and
Report on Internal Control Over Compliance Required by the Uniform Guidance**

Independent Auditor's Report

To the Management and Audit Committee of the Board of Directors
Memorial Hermann Health System

Report of Independent Auditors on Compliance for Each Major Federal Program

Opinion on Each Major Federal Program

We have audited Memorial Hermann Health System's (the "Health System") compliance with the types of compliance requirements identified as subject to audit in the U.S. Office of Management and Budget (OMB) *Compliance Supplement* that could have a direct and material effect on each of the Health System's major federal programs for the year ended June 30, 2024. The Health System's major federal programs are identified in the summary of auditor's results section of the accompanying schedule of findings and questioned costs.

In our opinion, the Health System complied, in all material respects, with the compliance requirements referred to above that could have a direct and material effect on each of its major federal programs for the year ended June 30, 2024.

Basis for Opinion on Each Major Federal Program

We conducted our audit of compliance in accordance with auditing standards generally accepted in the United States of America (U.S. GAAS); the standards applicable to financial audits contained in *Government Auditing Standards*, issued by the Comptroller General of the United States (*Government Auditing Standards*); and the audit requirements of Title 2 U.S. Code of Federal Regulations Part 200, *Uniform Administrative Requirements, Cost Principles, and Audit Requirements for Federal Awards* (Uniform Guidance). Our responsibilities under those standards and the Uniform Guidance are further described in the Auditors' Responsibilities for the Audit of Compliance section of our report.

We are required to be independent of the Health System and to meet our other ethical responsibilities, in accordance with relevant ethical requirements relating to our audit. We believe that the audit evidence we have obtained is sufficient and appropriate to provide a basis for our opinion on compliance for each major federal program. Our audit does not provide a legal determination of the Health System's compliance with the compliance requirements referred to above.

Responsibilities of Management for Compliance

Management is responsible for compliance with the requirements referred to above and for the design, implementation, and maintenance of effective internal control over compliance with the requirements of laws, statutes, regulations, rules and provisions of contracts or grant agreements applicable to the Health System's federal programs.



Auditor's Responsibilities for the Audit of Compliance

Our objectives are to obtain reasonable assurance about whether material noncompliance with the compliance requirements referred to above occurred, whether due to fraud or error, and to express an opinion on the Health System's compliance based on our audit. Reasonable assurance is a high level of assurance but is not absolute assurance and therefore is not a guarantee that an audit conducted in accordance with U.S. GAAS, *Government Auditing Standards*, and the Uniform Guidance will always detect material noncompliance when it exists. The risk of not detecting material noncompliance resulting from fraud is higher than that resulting from error, as fraud may involve collusion, forgery, intentional omissions, misrepresentations, or the override of internal control. Noncompliance with the compliance requirements referred to above is considered material, if there is a substantial likelihood that, individually or in the aggregate, it would influence the judgment made by a reasonable user of the report on compliance about the Health System's compliance with the requirements of each major federal program as a whole.

In performing an audit in accordance with U.S. GAAS, *Government Auditing Standards*, and the Uniform Guidance, we:

- Exercise professional judgment and maintain professional skepticism throughout the audit.
- Identify and assess the risks of material noncompliance, whether due to fraud or error, and design and perform audit procedures responsive to those risks. Such procedures include examining, on a test basis, evidence regarding the Health System's compliance with the compliance requirements referred to above and performing such other procedures as we considered necessary in the circumstances.
- Obtain an understanding of the Health System's internal control over compliance relevant to the audit in order to design audit procedures that are appropriate in the circumstances and to test and report on internal control over compliance in accordance with the Uniform Guidance, but not for the purpose of expressing an opinion on the effectiveness of the Health System's internal control over compliance. Accordingly, no such opinion is expressed.

We are required to communicate with those charged with governance regarding, among other matters, the planned scope and timing of the audit and any significant deficiencies and material weaknesses in internal control over compliance that we identified during the audit.

Report on Internal Control over Compliance

A *deficiency in internal control over compliance* exists when the design or operation of a control over compliance does not allow management or employees, in the normal course of performing their assigned functions, to prevent, or detect and correct, noncompliance with a type of compliance requirement of a federal program on a timely basis. A *material weakness in internal control over compliance* is a deficiency, or a combination of deficiencies, in internal control over compliance, such that there is a reasonable possibility that material noncompliance with a type of compliance requirement of a federal program will not be prevented, or detected and corrected, on a timely basis. A *significant deficiency in internal control over compliance* is a deficiency, or a combination of deficiencies, in internal control over compliance with a type of compliance requirement of a federal program that is less severe than a material weakness in internal control over compliance, yet important enough to merit attention by those charged with governance.



McConnell Jones

Our consideration of internal control over compliance was for the limited purpose described in the Auditor's Responsibilities for the Audit of Compliance section above and was not designed to identify all deficiencies in internal control over compliance that might be material weaknesses or significant deficiencies in internal control over compliance. Given these limitations, during our audit we did not identify any deficiencies in internal control over compliance that we consider to be material weaknesses, as defined above. However, material weaknesses or significant deficiencies in internal control over compliance may exist that were not identified.

Our audit was not designed for the purpose of expressing an opinion on the effectiveness of internal control over compliance. Accordingly, no such opinion is expressed.

The purpose of this report on internal control over compliance is solely to describe the scope of our testing of internal control over compliance and the results of that testing based on the requirements of the Uniform Guidance. Accordingly, this report is not suitable for any other purpose.

McConnell Jones LLP

Houston, Texas
March 25, 2025

MEMORIAL HERMANN HEALTH SYSTEM
SCHEDULE OF EXPENDITURES OF FEDERAL AWARDS
YEAR ENDED JUNE 30, 2024

Federal Grantor/Pass-Through Grantor/Program Title	Federal Assistance Listing Number	Federal Award Identifying Number	Pass-Through Award Identifying Number	Federal Award Expenditures	Passed Through to Subrecipients
U.S. Department of Health and Human Services					
Direct Agency Funded Programs:					
ACL National Institute on Disability, Independent Living, and Rehabilitation Research *	93.433	90DPAD0010	-	\$ 1,008,820	\$ 202,329
ACL National Institute on Disability, Independent Living, and Rehabilitation Research *	93.433	90DPTB0016	-	40,342	-
ACL National Institute on Disability, Independent Living, and Rehabilitation Research *	93.433	90DPAD0001	-	207,511	140,774
ACL National Institute on Disability, Independent Living, and Rehabilitation Research *	93.433	90DPTB0025	-	409,255	58,678
ACL National Institute on Disability, Independent Living, and Rehabilitation Research *	93.433	90SIMS0019	-	522,464	34,489
ACL National Institute on Disability, Independent Living, and Rehabilitation Research *	93.433	90DPAD0013	-	165,257	132,830
				<u>2,353,649</u>	<u>569,100</u>
Substance Abuse and Mental Health Services Projects of Regional and National Significance	93.243	H79SM084831	-	540,142	-
ACL Independent Living State Grants	93.369	90ISTA0002	-	274,596	67,206
ACL Centers for Independent Living	93.432	90ILTA0002	-	991,743	395,420
Total Direct Agency Funded Programs				<u>\$ 4,160,130</u>	<u>\$ 1,031,726</u>
Passed-Through Programs:					
<i>Passed-Through US Aging:</i>					
Special Programs for the Aging, Title IV, and Title II, Discretionary Projects *	93.048	90NWCH0001	-	\$ 4,457	\$ -
COVID-19 - Special Programs for the Aging, Title IV, and Title II, Discretionary Projects *	93.048	90EEC50001	-	21,630	-
COVID-19 - Special Programs for the Aging, Title IV, and Title II, Discretionary Projects *	93.048	90HDRC0007	-	312,064	-
				<u>338,151</u>	<u>-</u>

See accompanying notes to the schedule of expenditures of federal awards.

MEMORIAL HERMANN HEALTH SYSTEMS
SCHEDULE OF EXPENDITURES OF FEDERAL AWARDS (CONTINUED)
YEAR ENDED JUNE 30, 2024

Federal Grantor/Pass-Through Grantor/Program Title	Federal Assistance Listing Number	Federal Award Identifying Number	Pass-Through Award Identifying Number	Federal Award Expenditures	Passed Through to Subrecipients
<i>Passed-Through Indiana University:</i>					
ACL National Institute on Disability, Independent Living, and Rehabilitation Research *	93.433	90DPHF0006	-	21,171	-
ACL National Institute on Disability, Independent Living, and Rehabilitation Research *	93.433	90DPTB0035	9825	23,179	-
ACL National Institute on Disability, Independent Living, and Rehabilitation Research *	93.433	90DPTB0035	9927	82	-
				<u>44,432</u>	<u>-</u>
<i>Passed-Through University of Houston:</i>					
ACL National Institute on Disability, Independent Living, and Rehabilitation Research *	93.433	90IFST0006	R-22-0078	32,679	-
<i>Passed-Through University of Delaware:</i>					
ACL National Institute on Disability, Independent Living, and Rehabilitation Research *	93.433	90DPCP0010	UDR0000137	78,176	36,077
<i>Passed-Through Research Foundation of CUNY on behalf of Hunter College:</i>					
ACL National Institute on Disability, Independent Living, and Rehabilitation Research *	93.433	90DPGE0013	M00003839- 03/04	100,076	-
				<u>255,363</u>	<u>36,077</u>
<i>Passed-Through University of Delaware:</i>					
Nursing Research *	93.361	R01NR018684	55381	58,770	-
<i>Passed-Through Baylor College of Medicine:</i>					
Nursing Research *	93.361	5R01NR013658	-	60,279	-
				<u>119,049</u>	<u>-</u>
<i>Passed-Through University of Cincinnati:</i>					
Extramural Research Programs in the Neurosciences and Neurological Disorders *	93.853	U01NS110772	012765-140423	47,716	-
Extramural Research Programs in the Neurosciences and Neurological Disorders *	93.853	U01NS110772 U01NS110772	012765-00025 012765-00032	55,056	-
				<u>102,772</u>	<u>-</u>

See accompanying notes to the schedule of expenditures of federal awards.

MEMORIAL HERMANN HEALTH SYSTEMS
SCHEDULE OF EXPENDITURES OF FEDERAL AWARDS (CONTINUED)
YEAR ENDED JUNE 30, 2024

Federal Grantor/Pass-Through Grantor/Program Title	Federal Assistance Listing Number	Federal Award Identifying Number	Pass-Through Award Identifying Number	Federal Award Expenditures	Passed Through to Subrecipients
<i>Passed-Through The Ohio State University:</i>					
Extramural Research Programs in the Neurosciences and Neurological Disorders *	93.853	UG3NS117844	GR128734 I SPC- 1000006866 GR133232 I SPC- 1000012424	117,321	-
<i>Passed-Through University of Michigan:</i>					
Extramural Research Programs in the Neurosciences and Neurological Disorders *	93.853	U01NS099046	SUBK00015080	22,997	-
<i>Passed-Through Northwestern University:</i>					
Extramural Research Programs in the Neurosciences and Neurological Disorders *	93.853	R01NS110779	60052911 MHHS	13,494	-
<i>Passed-Through Washington University:</i>					
Extramural Research Programs in the Neurosciences and Neurological Disorders *	93.853	U01NS100699	WU-22-0134- MOD-4 WU-22-0134- MOD-5	33,776	-
				<u>290,360</u>	<u>-</u>
<i>Passed-Through University of Texas Health Science Center, Houston:</i>					
Research and Training in Complementary and Alternative Medicine *	93.213	R34AT011530	SA0002872	24,813	-
<i>Passed-Through American Association on Health and Disability:</i>					
Trans-NIH Research Support *	93.310	OT2OD031916	202306	3,109	-
<i>Passed Through CDC Foundation:</i>					
<i>Passed-Through Able South Carolina:</i>					
Strengthening Public Health Systems and Services through National Partnerships to Improve and Protect the Nation's Health *	93.421	NU38OT000288	-	50,504	-
Total Passed-Through Programs				<u>\$ 1,081,349</u>	<u>\$ 36,077</u>
Total U.S. Department of Health and Human Services				<u>\$ 5,241,479</u>	<u>\$ 1,067,803</u>

See accompanying notes to the schedule of expenditures of federal awards.

MEMORIAL HERMANN HEALTH SYSTEMS
SCHEDULE OF EXPENDITURES OF FEDERAL AWARDS (CONTINUED)
YEAR ENDED JUNE 30, 2024

Federal Grantor/Pass-Through Grantor/Program Title	Federal Assistance Listing Number	Federal Award Identifying Number	Pass-Through Award Identifying Number	Federal Award Expenditures	Passed Through to Subrecipients
U.S. Department of Defense					
Direct Agency Funded Programs:					
Military Medical Research and Development *	12.420	HT94252310567	-	\$ 218,070	\$ 158,049
Military Medical Research and Development *	12.420	HT94252310396	-	126,350	19,017
Total Direct Agency Funded Programs				\$ 344,420	\$ 177,066
Passed-Through Programs:					
<i>Passed through Baylor College of Medicine:</i>					
Military Medical Research and Development *	12.420	W81XWH1910735	-	\$ 1,559	\$ -
<i>Passed through Icahn School of Medicine at Mount Sinai:</i>					
Military Medical Research and Development *	12.420	W81XWH2210999	0258-A921-4605	1,791	-
<i>Passed through University of Texas Health Science Center:</i>					
Military Medical Research and Development *	12.420	W81XWH2210848	SA0002161	55,917	-
				<u>59,267</u>	<u>-</u>
<i>Passed through Medical Technology Enterprise Consortium</i>					
<i>Passed through University of California, San Francisco:</i>					
Basic Scientific Research *	12.431	-	A131954	1,253	-
Total Passed-Through Programs				\$ 60,520	\$ -
Total U.S. Department of Defense				\$ 404,940	\$ 177,066

See accompanying notes to the schedule of expenditures of federal awards.

MEMORIAL HERMANN HEALTH SYSTEMS
SCHEDULE OF EXPENDITURES OF FEDERAL AWARDS (CONTINUED)
YEAR ENDED JUNE 30, 2024

Federal Grantor/Pass-Through Grantor/Program Title	Federal Assistance Listing Number	Federal Award Identifying Number	Pass-Through Award Identifying Number	Federal Award Expenditures	Passed Through to Subrecipients
U.S. Department of Homeland Security					
<i>Passed through Texas Division of Emergency Management:</i>					
COVID-19 - Disaster Grants – Public Assistance (Presidentially Declared Disasters)	97.036	4485DRTXP000 0001	-	\$ 97,202,454	\$ -
Total Passed-Through Programs				\$ 97,202,454	\$ -
Total U.S. Department of Homeland Security				\$ 97,202,454	\$ -
Total Federal Expenditures				\$102,848,873	\$ 1,244,869

Cluster Title:

* Represents Research and Development
Cluster

Total Expenditures:

R&D cluster - \$3,839,938

ALN 12.420 - \$403,687

ALN 93.433 - \$2,609,012

See accompanying notes to the schedule of expenditures of federal awards.

MEMORIAL HERMANN HEALTH SYSTEM
NOTES TO THE SCHEDULE OF EXPENDITURES OF FEDERAL AWARDS
YEAR ENDED JUNE 30, 2024

1. ORGANIZATION AND BUSINESS

Memorial Hermann Health System (the Health System), a Texas nonprofit corporation, operates, directly and through affiliates, a multi-hospital, comprehensive health care system that has the largest market share in Houston, Texas, greater metropolitan area. The Health System's Board of Directors exercises governance control for the Health System and retains significant reserved powers regarding its affiliates. The Health System is exempt from federal income tax under Section 501(c)(3) of the Internal Revenue Code.

The Health System currently operates 17 hospitals (including 11 general acute care hospitals, one children's hospital, one orthopedic and spine hospital, two rehabilitation hospitals, and two surgical hospitals), a comprehensive network of ambulatory care facilities, physician organizations and an independent practice association, a health plan that underwrites group health coverage, a captive casualty and liability insurance company, and an accountable care organization. Additionally, the Health System is supported by the Memorial Hermann Foundation.

2. FINANCIAL STATEMENTS

The consolidated financial statements of the Health System include the accounts of its controlled affiliates, and all significant intercompany accounts and transactions have been eliminated. The consolidated financial statements of the Health System were audited by Ernst & Young, LLP (E&Y), whose report dated September 25, 2024, except for the schedule of expenditures of federal (the "Schedule") for which the date is March 25, 2025, was furnished to McConnell & Jones LLP. Such report indicated that the Schedule was subjected to the auditing procedures applied in E&Y's audit of the consolidated financial statements, and that the Schedule is fairly stated, in all material respects, in relation to the consolidated financial statements as a whole.

3. BASIS OF PRESENTATION

The accompanying Schedule presents federal grant expenditures of the Health System under programs of the federal governments for the year ended June 30, 2024. The information in this Schedules is presented in accordance with the requirements of Title 2 U.S. Code of Federal Regulations (CFR) Part 200, *Uniform Administrative Requirements, Cost Principles, and Audit Requirements for Federal Awards* (Uniform Guidance). Because the Schedule presents only a selected portion of the operations of the Health System, the Schedule is not intended to and does not present the financial position, changes in net assets, or cash flows of the Health System.

4. SUMMARY OF SIGNIFICANT ACCOUNTING POLICIES

Expenditures on the Schedule is generally reported on the accrual basis of accounting in accordance with the accounting principles generally accepted in the United States of America. Such expenditures also follow the cost principles contained in Subpart E of the Uniform Guidance wherein certain types of expenditures are not allowable or are limited as to reimbursement.

MEMORIAL HERMANN HEALTH SYSTEM
NOTES TO THE SCHEDULE OF EXPENDITURES OF FEDERAL AWARDS
YEAR ENDED JUNE 30, 2024

5. INDIRECT COST RATE

The Health System has not elected to use the 10% de minimis indirect cost rate for the year ended June 30, 2024, as allowed under the Uniform Guidance, since it either charges indirect costs specifically approved in a grant award document or, when allowed, charges an indirect cost rate previously approved from the U.S. Department of Health and Human Services, its oversight agency.

6. RELATIONSHIP TO FINANCIAL REPORTS SUBMITTED TO GRANTOR AGENCIES

Amounts reflected in the financial reports submitted to grantor agencies for the programs and the Schedule may not agree because of accruals which would be included in the next reports filed with grantor agencies and because of the different program year-ends.

7. FEDERAL DISASTER GRANT ASSISTANCE

During the COVID-19 pandemic, the Health System incurred costs associated with taking emergency protective measures during the period between January 20, 2020, through May 11, 2023, which were eligible for reimbursement under Federal Emergency Management Administration's (FEMA) Public Assistance (Presidentially Declared Disasters) (Assistance Listing No. 97.036). During fiscal year 2024, the Health System received FEMA's approval for \$97,202,454 which were reported in the Schedule as of June 30, 2024, but were incurred in prior fiscal years. However, additional expenditures in the amount of \$90,790,429 incurred during the COVID-19 pandemic in previous fiscal years were not reported in the Schedule since they were yet to be approved and obligated by FEMA as of June 30, 2024.

8. CONTINGENCIES

The Health System's federal programs are subject to review and audit by grantor agencies. Consequently, the Health System may become liable to refund money to funding agencies where it fails to comply with contractual provisions. In addition, the Health System may not fully collect federal grant receivables as of June 30, 2024, related to the reported federal grant expenditures as these receivables may be subject to the grantors' compliance approval process. Management believes that the results of these reviews and audits will not have a material effect on the amounts reported in the Schedule.

MEMORIAL HERMANN HEALTH SYSTEM
SCHEDULE OF FINDINGS AND QUESTIONED COSTS
YEAR ENDED JUNE 30, 2024

PART I – SUMMARY OF AUDITOR’S RESULTS

Financial Statement Section

- | | |
|---|------------|
| 1. Type of auditor’s report issued: | Unmodified |
| 2. Internal control over financial reporting: | |
| a) Material weaknesses identified? | No |
| b) Significant deficiencies identified, which are not considered to be material weaknesses? | No |
| 3. Noncompliance material to financial statements noted? | No |

Federal Awards Section

- | | |
|---|----|
| 1. Internal control over major programs: | |
| a) Material weaknesses identified? | No |
| b) Significant deficiencies identified, which are not considered to be material weaknesses? | No |
| 2. Type of auditor’s report issued on compliance for major programs: | No |
| 3. Any audit findings disclosed, which are required to be reported in accordance with 2 CFR Section 200.516(a)? | No |
| 4. Identification of major programs: | |

	<u>Assistance</u>
	<u>Listing</u>
	<u>Number</u>
<u>Name of Federal Programs</u>	
Research and Development Cluster	Various
COVID-19 - Disaster Grants – Public Assistance (Presidentially Declared Disasters)	97.036
5. Dollar threshold used to distinguish between type A and B programs:	\$3,000,000
6. Auditee qualified as low-risk auditee under 2 CFR Section 200.520?	No

**MEMORIAL HERMANN HEALTH SYSTEM
SCHEDULE OF FINDINGS AND QUESTIONED COSTS
YEAR ENDED JUNE 30, 2024**

PART II – FINDINGS - FINANCIAL STATEMENT AUDIT

None noted.

**MEMORIAL HERMANN HEALTH SYSTEM
SCHEDULE OF FINDINGS AND QUESTIONED COSTS
YEAR ENDED JUNE 30, 2024**

**PART III – FINDINGS AND QUESTIONED COSTS - MAJOR FEDERAL AWARD
PROGRAMS**

None noted.

MEMORIAL HERMANN HEATH SYSTEM
SUMMARY SCHEDULE OF PRIOR AUDIT FINDINGS
YEAR ENDED JUNE 30, 2024

The following provides current status of audit finding previously reported in the fiscal year ended June 30, 2023, audit:

Federal Award Information – Research and Development Cluster (Assistance Listing Nos. 93.432, 93.369, 93.433, and 93.421)

Finding 2023-001 – Procurement and Suspension and Debarment

Condition: The Health System did not maintain records for procurements made with federal funds sufficient to detail the history of procurement, including the rationale for the method of procurement, selection of contract type, contractor selection or rejection, and the basis for the contract price.

Recommendation: The Health System should retain written documentation for procurements documenting the history of the procurement prior to the procurement of goods or services including, but not limited to, the rationale for the method of procurement, selection of contract type, contractor selection or rejection, and the basis for the contract price. This would include obtaining quotes from vendors for competitive procurements and documenting sole source justification for procurements, when applicable.

Current Status: *The Health System remediated this finding and implemented its corrective action plan by updating its procurement policy on federally funded awards requiring use of a standard form to document justification of sole/single source purchases. As such, this finding was resolved.*